Application Submitted to HRSA

Submitted to HRSA

Organization: WEST VIRGINIA UNIVERSITY RESEARCH CORPORATION, MORGANTOWN, West Virginia

Grants.gov Tracking Number: GRANT14708969

EHB Application Number: 248134

Grant Number: 4 T73MC00043-33-03

Funding Opportunity Number: HRSA-26-019

Received Date: 7/10/2026 4:41:26 PM

A printer version document only. The document may contain some accessibility challenges for the screen reader users. To access same information, a fully 508 compliant accessible HTML version is available on the HRSA Electronic Handbooks. If you need more information, please contact HRSA contact center at 877-464-4772, 8 am to 8 pm ET, weekdays.

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OMB Number: 4040-0001
Expiration Date: 01/31/2029

APPLICATION FOR FEDERAL ASSISTANCE
SF 424 (R&R)

3. DATE RECEIVED BY STATE

State Application Identifier

1. TYPE OF SUBMISSION

☐ Pre-application ☒ Application ☐ Changed/Corrected Application

2. DATE SUBMITTED

Applicant Identifier

4. a. Federal Identifier

b. Agency Routing Identifier

c. Previous Grants.gov Tracking ID

5. APPLICANT INFORMATION

UEI: M7PNRH24BBM8

Legal Name: West Virginia University Research Corporation

Department:

Division:

Street1: 886 Chestnut Ridge Rd.

Street2: PO Box 6845

City: Morgantown County / Parish:

State: WV: West Virginia Province:

Country: USA: UNITED STATES ZIP / Postal Code: 265066845

Person to be contacted on matters involving this application

Prefix: Dr. First Name: Cassaundra Middle Name:

Last Name: Miller Suffix: EdD

Position/Title: WV LEND Director/Assistant Professor SOM

Street1: 959 Hartman Run Road

Street2: PO Box 6870

City: Morgantown County / Parish: WV

State: WV: West Virginia Province:

Country: USA: UNITED STATES ZIP / Postal Code: 265066870

Phone Number: (304) 293-4692 Fax Number:

Email: ccorbett@hsc.wvu.edu

6. EMPLOYER IDENTIFICATION *(EIN) or (TIN):* 550665758

7. TYPE OF APPLICANT: M: Nonprofit with 501C3 IRS Status (Other than Institution of Higher Education)

Other (Specify):

Small Business Organization Type ☐ Women Owned ☐ Socially and Economically Disadvantaged

8. TYPE OF APPLICATION:

☒ New ☐ Resubmission

☐ Renewal ☐ Continuation ☐ Revision

If Revision, mark appropriate box(es).

☐ A. Increase Award ☐ B. Decrease Award ☐ C. Increase Duration ☐ D. Decrease Duration

☐ E. Other (specify):

Is this application being submitted to other agencies? Yes ☐ No ☒ What other Agencies?

9. NAME OF FEDERAL AGENCY: Health Resources and Services Administration

10. ASSISTANCE LISTING NUMBER: 93.877

ASSISTANCE LISTING TITLE: Autism Collaboration, Accountability, Research, Education, and Support

11. DESCRIPTIVE TITLE OF APPLICANT'S PROJECT:

Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) WV

12. PROPOSED PROJECT:

Start Date: 09/30/2026 Ending Date: 06/30/2031

13. CONGRESSIONAL DISTRICT OF APPLICANT

WV-002

14. PROJECT DIRECTOR/PRINCIPAL INVESTIGATOR CONTACT INFORMATION

Prefix: Dr. First Name: Cassaundra Middle Name:

Last Name: Miller Suffix: EdD

Position/Title: Assistant Professor SOM

Organization Name: West Virginia University Ctr for Excellence in Disabilities

Department: Disability Studies

Division:

Street1: 959 Hartman Run Road

Street2: Po Box 6870

City: Morgantown County / Parish: WV

State: WV: West Virginia Province:

Country: USA: UNITED STATES ZIP / Postal Code: 265066870

Phone Number: 304-293-4692 Fax Number:

Email: ccorbett@hsc.wvu.edu

15. ESTIMATED PROJECT FUNDING

a. Total Federal Funds Requested	3,132,840.00
b. Total Non-Federal Funds	0.00
c. Total Federal & Non-Federal Funds	3,132,840.00
d. Estimated Program Income	0.00

16. IS APPLICATION SUBJECT TO REVIEW BY STATE EXECUTIVE ORDER 12372 PROCESS?

a. YES ☐ THIS PREAPPLICATION/APPLICATION WAS MADE AVAILABLE TO THE STATE EXECUTIVE ORDER 12372 PROCESS FOR REVIEW ON:

DATE:

b. NO ☒ PROGRAM IS NOT COVERED BY E.O. 12372; OR

☐ PROGRAM HAS NOT BEEN SELECTED BY STATE FOR REVIEW

17. By signing this application, I certify (1) to the statements contained in the list of certifications* and (2) that the statements herein are true, complete and accurate to the best of my knowledge. I also provide the required assurances * and agree to comply with any resulting terms if I accept an award. I am aware that any false, fictitious. or fraudulent statements or claims may subject me to criminal, civil, or administrative penalties. (U.S. Code, Title 18, Section 1001)

☒ **I agree**

**The list of certifications and assurances, or an Internet site where you may obtain this list, is contained in the announcement or agency specific instructions.*

18. SFLLL (Disclosure of Lobbying Activities) or other Explanatory Documentation

Add Attachment | Delete Attachment | View Attachment

19. Authorized Representative

Prefix: First Name: Cathy Middle Name:

Last Name: Tarabrella Suffix:

Position/Title: Associate Director of OSP, Pre-Award

Organization: West Virginia University Research Corporation

Department: Office of Sponsored Programs

Division:

Street1: 886 Chestnut Ridge Rd.

Street2: PO Box 6845

City: Morgantown County / Parish:

State: WV: West Virginia Province:

Country: USA: UNITED STATES ZIP / Postal Code: 265066845

Phone Number: (304) 293-3998 Fax Number: (304) 293-7435

Email: wvusponsoredprograms@mail.wvu.edu

Signature of Authorized Representative	Date Signed
Cathy.Tarabrella	07/10/2026

20. Pre-application Add Attachment | Delete Attachment | View Attachment

21. Cover Letter Attachment Add Attachment | Delete Attachment | View Attachment

OMB No.: 0915-0285. Expiration Date: 04/30/2026

DEPARTMENT OF HEALTH AND HUMAN SERVICES Health Resources and Services Administration Equipment List (AS APPLICABLE)	FOR HRSA USE ONLY	
	Grant Number	Application Tracking Number

Instructions

Equipment costs entered here should be consistent with those provided in the Budget Narrative and SF-424A Budget Information Form. Equipment means tangible personal property (including information technology systems) having a useful life of more than one year and a per-unit acquisition cost which equals or exceeds $10,000. Equipment that does not meet the $10,000 threshold should be considered supplies and should not be entered on this form.

List of Equipment

Type	Description	Unit Price	Quantity	Total Price
Grand Total			**0**	**$0.00**

Public Burden Statement: Health centers (section 330 grant funded and Federally Qualified Health Center look-alikes) deliver comprehensive, high quality, cost-effective primary health care to patients regardless of their ability to pay. The Health Center Program application forms provide essential information to HRSA staff and objective review committee panels for application evaluation; funding recommendation and approval; designation; and monitoring. The OMB control number for this information collection is 0915-0285 and it is valid until 4/30/2026. This information collection is mandatory under the Health Center Program authorized by section 330 of the Public Health Service (PHS) Act (42 U.S.C. 254b). Public reporting burden for this collection of information is estimated to average 30 minutes per response, including the time for reviewing instructions, searching existing data sources, and completing and reviewing the collection of information. Send comments regarding this burden estimate or any other aspect of this collection of information, including suggestions for reducing this burden, to HRSA Reports Clearance Officer, 5600 Fishers Lane, Room 14N136B, Rockville, Maryland, 20857 or paperwork@hrsa.gov.

OMB Number: 4040-0019
Expiration Date: 06/30/2028

Project Abstract Summary

This Project Abstract Summary form must be submitted or the application will be considered incomplete. Ensure the Project Abstract field succinctly describes the project in plain language that the public can understand and use without the full proposal. Use 4,000 characters or less. Do not include personally identifiable, sensitive or proprietary information. Refer to Agency instructions for any additional Project Abstract field requirements. If the application is funded, your project abstract information (as submitted) will be made available to public websites and/or databases including USAspending.gov.

Funding Opportunity Number

HRSA-26-019

Assistance Listing Number(s):

93.877

Applicant Name

West Virginia University Research Corporation

Descriptive Title of Applicant's Project

Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND) WV

Project Abstract

Purpose and Outcomes
The West Virginia Leadership Education in Neurodevelopmental and Related Disabilities (WV LEND) Program develops interdisciplinary leaders grounded in Maternal Child Health principles of family-centered, community-based, and systems of care-focused services for children and youth with autism/DD. Through leadership training, clinical/community experiences, and academic engagement, WV LEND prepares individuals with disabilities, family members of individuals with disabilities, students, and practicing professional to improve outcomes for children and youth with autism/DD and their families. Through new and longstanding partnerships, WV LEND expands the interdisciplinary workforce to address complex needs of individuals with autism.DD in West Virginia’s predominantly rural communities.

Design, Management, Outcomes
WV LEND trainees participate in a comprehensive curriculum combining academic, clinical, leadership, and community-based experiences to master competencies addressing the complex needs of children and youth with autism/DD across their lifespan and their families. Continuing education opportunities will advance the knowledge, skills, and leadership of practicing professionals. Training outcomes are monitored through an established online evaluation system to support continuous quality improvement. By expanding workforce capacity, fostering interdisciplinary collaborations, strengthening family partnerships, and advancing systems improvement, WV LEND increases access to evidence-based screening, evaluation, diagnosis, and intervention for individuals with autism and their families.

OMB Number: 4040-0001
Expiration Date: 01/31/2029

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Project Director/Principal Investigator

Prefix: Dr.
* First Name: Cassaundra
Middle Name:
* Last Name: Miller
Suffix: EdD
Position/Title: Assistant Professor SOM
Department: Disability Studies
Organization Name: West Virginia University Ctr for Excellence in Disabilities
Division:
* Street1: 959 Hartman Run Road
Street2: Po Box 6870
* City: Morgantown
County/ Parish: WV
* State: WV: West Virginia
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 265066870
* Phone Number: 304-293-4692
Fax Number:
* E-Mail: ccorbett@hsc.wvu.edu

Credential, e.g., agency login:

*** Project Role:** PD/PI
Other Project Role Category:

Degree Type: EdD
Degree Year: 2021

***Attach Biographical Sketch**	Corbett_Miller_biosketch.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

PROFILE - Senior/Key Person 1

Prefix: Dr.
* First Name: Margaret
Middle Name:
* Last Name: Jaynes
Suffix: MD
Position/Title: West Virginia LEND Co-PD/PI
Department: Pediatrics (Medicine Department)
Organization Name: WVU School of Medicine
Division: Pediatrics
* Street1: 1 Medical Center Drive
Street2: PO BOX 9214
* City: Morgantown
County/ Parish:
* State: WV: West Virginia
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 265060000
* Phone Number: 304-293-4451
Fax Number:
* E-Mail: mjaynes@hsc.wvu.edu

Credential, e.g., agency login:

*** Project Role:** Co-PD/PI
Other Project Role Category:

Degree Type: MD
Degree Year: 1991

Attach Biographical Sketch	Jaynes_biosketch.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

RESEARCH & RELATED Senior/Key Person Profile (Expanded)

PROFILE - Senior/Key Person 2

Prefix: Dr.
* First Name: Amy
Middle Name:
* Last Name: Burt
Suffix: MD
Position/Title: Professor
Department: Disability Studies
Organization Name: WVUCED
Division:
* Street1: 959 Hartman Run Road
Street2:
* City: Morgantown
County/ Parish: WV
* State: WV: West Virginia
Province:
* Country: USA: UNITED STATES
* Zip / Postal Code: 265050000
* Phone Number: 3042934266
Fax Number:
* E-Mail: alburt@hsc.wvu.edu

Credential, e.g., agency login:

*** Project Role:** Faculty
Other Project Role Category:

Degree Type: MD
Degree Year:

Attach Biographical Sketch	Burt_Biosketch_.pdf	Add Attachment	Delete Attachment	View Attachment
Attach Current & Pending Support		Add Attachment	Delete Attachment	View Attachment

OMB No. 0925-0001 and 0925-0002 (Rev. 10/2021 Approved Through 01/31/2026)

BIOGRAPHICAL SKETCH

Provide the following information for the Senior/key personnel and other significant contributors.
Follow this format for each person. **DO NOT EXCEED FIVE PAGES.**

NAME: Corbett Miller, Cassaundra

eRA COMMONS USER NAME (credential, e.g., agency login): ccorbett

POSITION TITLE: Assistant Professor

EDUCATION/TRAINING *(Begin with baccalaureate or other initial professional education, such as nursing, include postdoctoral training and residency training if applicable. Add/delete rows as necessary.)*

INSTITUTION AND LOCATION	DEGREE *(if applicable)*	Completion Date MM/YYYY	FIELD OF STUDY
Potomac State College of West Virginia University, Keyser, WV	AA	05/2002	General Studies
West Virginia University, Morgantown, WV	BS	05/2004	Speech Pathology & Audiology
West Virginia University, Morgantown, WV	MS	05/2006	Speech Pathology
Frostburg State University, Frostburg, MD	EdD	05/2021	Educational Leadership

A. Personal Statement

I am an Assistant Professor in the Department of Disability Studies at West Virginia University (WVU). I teach and develop curriculum focused on individuals with disabilities and how those individuals interact with their environment. I am also a practicing speech language pathologist with a focus on high intensity feeding and communication needs. I am the Director for the Feeding and Swallowing Clinic at the WVU Center for Excellence in Disabilities (CED). The Feeding Clinic at the CED is one of only two multidisciplinary feeding clinics in the state of WV. Because of the rurality of our state, our clinic families may drive up to five hours for an appointment. As a result, we have a strong telehealth network to offer hybrid treatment plans. Our teaching clinic includes myself as the SLP, OT, PT, psychology, social work, and dietetics along with up to 20 graduate level trainees. My background as an SLP includes over a decade of experience serving children with high intensity needs. During my time in early intervention, I was also part of transition planning for each child as they aged out of early intervention and into the WV public-school system. At the CED I am a clinical instructor for speech pathology graduate clinicians. I am also the Program Manager for the Leadership Education in Neurodevelopmental Disabilities (LEND) Program. LEND is an MCH-funded training program that brings together graduate and doctoral students, self-advocates, family members of individuals with disabilities, professional mentors, and community members to support advocacy and leadership in the field of disabilities. During my time at the CED, I have developed and implemented curriculum for multiple programs and projects, served on advisory boards, assisted with grant writing, gathered and analyzed program data to support grant goals and objectives. My professional experience up to this point has well prepared me to take on this proposed project to better outcomes for school-aged children with high-intensity needs across WV.

B. Positions, Scientific Appointments, and Honors

West Virginia University Center for Excellence in Disabilities (WVUCED), Morgantown, WV

2023 - Assistant Professor, WVU SOM, Professional Programs Clinical/Health Sciences Educator
2022 – Clinic Director, WVUCED
2015 - Speech Language Pathologist for WVUCED Feeding & Swallowing Clinic
2021 - Program Manager, WVUCED, Leadership Education in Neurodevelopmental Disabilities (LEND)
2009 – 2021 Pediatric Speech Language Pathologist West Virginia Birth to Three, Morgantown, WV
2006 – 2009 Pediatric/Pulmonary Speech Language Pathologist, HealthSouth Regional Rehabilitation Hospital

CERTIFICATIONS & AFFILIATIONS

◦ LEND Rural Workgroup, *2022-present*
◦ West Virginia Birth to Three Feeding Workgroup, *2021-2022*
◦ American Speech-Language-Hearing Association (ASHA), *2006-present*
◦ WV Board of Examiners Speech Language Pathology & Audiology, *2006-present*

C. Contributions to Science

WV Birth to Three Miller, PI 2022
Feeding intervention trends in West Virginia Birth to Three: A survey of speech language pathologists and occupational therapists

West Virginia University Miller, CoI 2022
Effects of Reinforcement-Based Intervention on Clients with Food Selectivity

WVU Miller, PI 2020
Clinical learning in higher education during the COVID-19 pandemic: Student comparison of in-person and telehealth teaching paradigms

WVU Miller, Study Personnel 2020
Perspectives on pediatric feeding and swallowing interventions: Survey of speech-language pathologists in West Virginia

PUBLICATIONS

Kurowski-Burt, A., & Corbett Miller, C. (2022). Strengths based approach to pediatric feeding. *AOTA Special Interest Section Quarterly*, 7(2).

Ray, C., Cottrell, L., & Corbett Miller, C. Speech language pathology perceptions in the time of COVID-19: Telehealth & dysphagia services in West Virginia. *West Virginia Medical Journal (WVMJ),* June 2021.

GRANTS & AWARDS

2022 – 2025 **American Speech Language Hearing Association (ASHA) Award for Continuing Education**
2022-2024 **WVU Faculty Senate Community Engagement Grant,** *West Virginia University,* Burt A. & Corbett Miller C., "Sensory and Oral Motor Skills for Complex Feeding Disorders," $6512.39

CURRICULUM DEVELOPMENT

April 2023 **Conference Co-Developer,** WV LEND: Rural Wellness & Community Connections Workshop, *West Virginia University Center for Excellence in Disabilities, Morgantown, WV*
◦ 1-day hybrid conference with speakers and participants from 17 states
◦ Audience- 150 registered providers, community members, self-advocates, families, students, & faculty

Spring 2022 **Developer,** Disability Inclusion Identify Student Badge, *West Virginia University, Morgantown, WV*
◦ Provost Office certified micro-credential

Summer 2021 **Developer,** LEND Summer Institute, *West Virginia University Center for Excellence in Disabilities, Morgantown, WV*
◦ Weekly educational modules for LEND trainees with content contributions from LEND Mentors

OMB No. 3145-0279 (Expiration Date 10/31/2026)

NIH BIOGRAPHICAL SKETCH COMMON FORM

Name: Burt, Amy

Persistent Identifier (PID) of the Senior/Key Person: https://orcid.org/0000-0002-7120-2123

Position Title: Occupational Therapist

Organization and Location: West Virginia University Center for Excellence in Disabilities, Morgantown, West Virginia, United States

PROFESSIONAL PREPARATION

INSTITUTION AND LOCATION	DEGREE	Start Date	Completion Date	FIELD OF STUDY
West Virginia University, Morgantown, WV, United States	DOCTOR OF EDUCATION	08/2013	12/2019	Educational Psychology
University of Pittsburgh, Pittsburgh, PA, United States	OTHER MASTERS	06/2001	05/2003	Occupational Therapy
Washington & Jefferson College, Washington, PA, United States	BACHELOR OF ARTS	06/1997	05/2001	Biology

Appointments and Positions

2021 - present	Occupational Therapist, West Virginia University Center for Excellence in Disabilities, Morgantown, West Virginia, United States
2026 - present	Full Professor, West Virginia University, Morgantown, West Virginia, United States
2021 - 2023	Program Director, West Virginia University Masters of Occupational Therapy Program, Morgantown, West Virginia, United States
2019 - 2026	Associate Professor, West Virginia University, Morgantown, West Virginia, United States
2012 - 2019	Assistant Professor, West Virginia University, Morgantown, West Virginia, United States
2010 - 2012	Occupational Therapist, OSPTA/ ARS, Waynesburg, Pennsylvania, United States
2005 - 2010	Occupational Therapist, Children's Therapy Center of Washington Hospital, McMurray, Pennsylvania, United States

Products

Products Closely Related to the Proposed Project

1. Burt A, Miller C. Strengths-based approach for pediatric feeding. SIS Quarterly Practice Connections. 2022; 7(4). Available from: https://www.aota.org/publications/sis-quarterly/children-youth-sis/cysis-11-22
2. Scaife B, Burt A, Wheeler S, Levander H. Long term care and autism: A neurodiversity approach. SIS Quarterly Practice Connection. 2022; 7(3). Available from: https://www.aota.org/publications/sis-quarterly/productive-aging-sis/pasis-8-22
3. Sizemore JN, Kurowski-Burt A, Evans K, Hoffman A, Summers A, Baugh GM. Interdisciplinary Education Apartment Simulation (IDEAS) Project: An Interdisciplinary Simulation for Transitional Home Care. MedEdPORTAL. 2021 Feb 26;17:11111. PubMed Central PMCID: PMC7908376.
4. Webster PJ, Frum C, Kurowski-Burt A, Bauer CE, Wen S, Ramadan JH, Baker KA, Lewis JW. Processing of Real-World, Dynamic Natural Stimuli in Autism is Linked to Corticobasal Function. Autism Res. 2020 Apr;13(4):539-549. PubMed Central PMCID: PMC7418054.

Other Significant Products Highlighting Contributions to Science

1. Burt A, Haddox J. 202. https://doi.org/10.1177/0022487118803583 Burt, A. L., & Haddox, J. C. (2018). Barriers to health care participation in persons with disabilities in Appalachia: A qualitative pilot study. Health Environments Research & Design Journal. 2018; 11(4). Available from: https://doi.org/https://doi.org/10.1177/1937586718786127
2. Grygas-Coogle C, Storie S, Ottley J, Rahn N, Burt A. Performance-based Feedback to Enhance Pre-service Teachers' Practice and Preschool Children's Expressive Communication. Journal of Teacher Education. 2018; 7(2):188-202. Available from: https://doi.org/10.1177/0022487118803583

OMB No. 0925-0001 (Expiration Date 12/31/2027) and No. 0925-0002 (Expiration Date 11/30/2027)

NIH BIOGRAPHICAL SKETCH SUPPLEMENT

Name: Burt, Amy

Persistent Identifier (PID) of the Senior/Key Person: https://orcid.org/0000-0002-7120-2123

Position Title: Occupational Therapist

Organization and Location: West Virginia University Center for Excellence in Disabilities, Morgantown, West Virginia, United States

Personal Statement

I am a Professor at West Virginia University (WVU) in the Division of Disability Studies, and an occupational therapist for the WVU Center for Excellence in Disabilities. My research and clinical experiences are with children with disabilities, specifically autism and other developmental disorders, as well as working with adults with developmental disabilities. I have a broad background in pediatric occupational therapy, with specific training and expertise in sensory integration, behavioral management, feeding assessment and intervention, health and wellness, environmental modification including ADA and universal design, and development. Much of my clinical work serves children of diverse abilities focusing on community participation. I have served as a PI on numerous intramural university service grant opportunities to support these projects. In addition, I successfully administered the projects (e.g. staffing, research protections, budget), collaborated with other researchers, and disseminated the concepts about each project. This current project is congruent with the other initiatives I have focused on for years regarding training related service providers for evidence based and collaborative assessment and intervention techniques. In summary, I have the expertise, leadership, training, and motivation necessary to successfully carry out the proposed project.

Honors

2023 - 2023	Interprofessional Collaboration Award, American Occupational Therapy Association
2021 - 2021	Vice Presidents Award, Team Award, Go First Award, WVU HSC Vice President's Award
2021 - 2021	Academy of Excellence in Teaching and Learning, HSC Faculty Development
2019 - 2019	Distinguished Teacher Award, School of Medicine, WVU

Contributions to Science

1. AMCHP Replication Grant, "STAT-MD: Early Identification of ASD for Pediatric Health Providers" Project leader, 2026-2028, $25,000
2. ITAC/ AUCD grant, Burt, A. L. (Project Leader), 2026, “Disability Experience NDD Curriculum”, $5,861
3. ITAC/ AUCD Fast grant, Burt, A. L. (Project Leader), 2023-2024, “Lived Experience Advisory Committee”, $8,000
4. WVU Faculty Senate grant, Burt, A. L. (Project Leader), 2022-2024, “Sensory and Oral Motor Skills for Complex Feeding Disorders”, $6512.39

Certification:

BIOGRAPHICAL SKETCH

NAME: Margerat E. Jaynes, MD

eRA COMMONS USER NAME (credential, e.g., agency login): JAYNESM

POSITION TITLE: Professor Emeritus of Pediatrics & Neurology, Section Pediatric Neurology & Child Development

EDUCATION/TRAINING

INSTITUTION AND LOCATION	DEGREE	Completion Date	FIELD OF STUDY
West Virginia University, Morgantown, WV	BA	1975	Psycho-biology
Northwestern University, Evanston IL	MA	1976	Communication Disorders
West Virginia University, Morgantown, WV	MD	1991	Medicine
West Virginia University, Morgantown, WV		7/91-6/93	Pediatric Residency
West Virginia University, Morgantown, WV		7/93-6/96	Ped Neurology Residency

A.Personal Statement

I am a board certified pediatric neurologist. She is Professor Emritus of Pediatric Neurology and Child Development. She has advocated for the use of telehealth on both the university, state and national level. She has directed telehealth clinics for twenty-five years. She served on the WV Telehealth Alliance Board from 2010 through 2017 and was chairman from January 2014 to January 2015.She is an Advisory Board member to the Mid Atlantic Regional Telehealth Center. She has spoken at several national meetings on the use of telehealth in the pediatric specialty clinic. She serves on several telehealth committees for WVU Medicine including telehealth services within the Department of Pediatrics.

Dr. Jaynes has devoted her life to children with special healthcare needs. She started her career as a special education teacher and then after medical school became a pediatric neurologist. She has served on the Medical Advisory Board of the WV CSHCN program for approximately 20 years.

Dr. Jaynes is currently Project Director for the Paths for Parents, funded by WVCSHCN program. She was Principal Investigator of HRSA funded grants including Project Access Epilepsy Grant which focused on the use of telemedicine for patient care. She currently is the Co-Project Director for the HRSA funded Leadership Education in Neuro-Developmental and other related Disabilities Program (LEND) after having served as Project Director for 20 years. She was involved in a state funded project involving remote patient monitoring in the Aged and Disabled Waiver Program.

Positions and Honors

7/1996-6/2001	Assistant Professor, WVU Dept. Neurology and Pediatrics, Morgantown, WV
7/2001-6/2007	Associate Professor, WVU Dept. Neurology and Pediatrics, Morgantown, WV
6/1999-2021	Section Chief Pediatric Neurology, WVU, Morgantown, WV
12/2000-2021	Section Chief, Child Development, WVU Dept. of Pediatrics, Morgantown, WV
12/2000-Present	LEND Director, Center for Excellence in Disabilities, WVU, Morgantown, WV
7/2007-Present	Professor, WVU Dept. Neurology and Pediatrics, Morgantown, WV

B. Contributions to Science

1.Davis S, Jones A, Jaynes M, et al. Designing a multifaceted telehealth intervention for a rural population using model for developing complex interventions in nursing. BMC Nursing 2020;19.9.

2.Mallow J, Davis S, Herczyk J, Pauly N, Klos B, Jones A, Jaynes M, Theeke L(2121) Dose of Telehealth to Improve Community-based Care for Adults Living with Multiple Chronic Conditions: A Systematic Review. E-Health Telecommunication Systems and Networks, 10.20-39.

C. Additional Information: Research Support and/or Scholastic Performance

Leadership Education in Neurodevelopmental Disabilities 07/2025 – 09/2026
HRSA/Maternal Child Health Role: Co-Principal Investigator Percent Effort: 15%

Project/Performance Site Location(s)

Project/Performance Site Primary Location

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name: West Virginia University

UEI: MNL4D3VQUWA5

* Street1: 959 Hartman Run Road

Street2: PO Box 6870

* City: Morgantown

County: WV

* State: WV: West Virginia

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code: 265066870

* Project/ Performance Site Congressional District: WV-002

Project/Performance Site Location 1

☐ I am submitting an application as an individual, and not on behalf of a company, state, local or tribal government, academia, or other type of organization.

Organization Name:

UEI:

* Street1:

Street2:

* City:

County:

* State:

Province:

* Country: USA: UNITED STATES

* ZIP / Postal Code:

* Project/ Performance Site Congressional District:

Additional Location(s)

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 1

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: M7PNRH24BBM8 **Enter name of Organization:** West Virginia University Research Corporation

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 1 **Start Date:** 09/30/2026 **End Date:** 06/30/2027

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Cassaundra		Miller	EdD	70,480.00	7.80			34,418.00	9,121.00	43,539.00
Project Role:	PD/PI										
Dr.	Margaret		Jaynes	MD	228,000.00	1.80			25,650.00	2,001.00	27,651.00
Project Role:	Co-I										
Dr.	Amy		Burt	MD	111,789.00	6.00			41,921.00	11,107.00	53,028.00
Project Role:	Clinical Coordinator										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 124,218.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
2	Secretarial/Clerical	24.00			62,478.00	16,557.00	79,035.00
3	Communications/Dissemination	2.40			8,960.00	2,374.00	11,334.00
1	Data	1.20			5,400.00	1,431.00	6,831.00
2	Clinical Staff	9.84			16,615.00	1,296.00	17,911.00
3	Mentors	5.28			11,508.00	3,050.00	14,558.00
1	Finance	2.40			8,083.00	2,142.00	10,225.00
12	**Total Number Other Personnel**					**Total Other Personnel**	139,894.00

Total Salary, Wages and Fringe Benefits (A+B) 264,112.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file	
Total Equipment	

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	5,146.00
2.	Foreign Travel Costs	0.00
	Total Travel Cost	5,146.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	198,000.00
3.	Travel	
4.	Subsistence	
5.	Other	
18	**Number of Participants/Trainees** — **Total Participant/Trainee Support Costs**	198,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	11,778.00
2.	Publication Costs	0.00
3.	Consultant Services	66,316.00
4.	ADP/Computer Services	
5.	Subawards/Consortium/Contractual Costs	0.00
6.	Equipment or Facility Rental/User Fees	
7.	Alterations and Renovations	
8.	Printing/Binding/Postage	2,500.00
9.	Rent/Utilities/Telecommunications	47,000.00
10.	Hospitality	2,000.00
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	129,594.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	596,852.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	353,852.00	28,308.00
		Total Indirect Costs	28,308.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number) DoHS; POC Darryl W. Mayes (214) 767-3261

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	625,160.00

J. Fee

Funds Requested ($)
0.00

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	625,160.00

L. Budget Justification

(Only attach one file.) 2_0_Budget_Justification_5YR_LEND_Comp

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 2

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: M7PNRH24BBM8 **Enter name of Organization:** West Virginia University Research Corporation

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 2 **Start Date:** 07/01/2027 **End Date:** 06/30/2028

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Cassaundra		Miller	EdD	70,660.00	7.80			45,929.00	12,171.00	58,100.00
Project Role:	PD/PI										
Dr.	Margaret		Jaynes	MD	228,000.00	0.60			11,400.00	889.00	12,289.00
Project Role:	Co-I										
Dr.	Amy		Burt	MD	111,849.00	5.40			50,332.00	13,338.00	63,670.00
Project Role:	Clinical Coordinator										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 134,059.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
2	Secretarial/Clerical	24.00			95,004.00	25,176.00	120,180.00
3	Communications	1.80			8,114.00	2,150.00	10,264.00
1	Data	1.20			7,218.00	1,913.00	9,131.00
2	Clinical Staff	8.64			16,304.00	1,272.00	17,576.00
3	Mentors	5.28			15,343.00	4,066.00	19,409.00
1	Finance	2.40			10,789.00	2,859.00	13,648.00
12	**Total Number Other Personnel**					**Total Other Personnel**	190,208.00

Total Salary, Wages and Fringe Benefits (A+B) 324,267.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	2,500.00
2.	Foreign Travel Costs	
	Total Travel Cost	2,500.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	176,000.00
3.	Travel	
4.	Subsistence	
5.	Other	
	Total Participant/Trainee Support Costs	176,000.00

18 **Number of Participants/Trainees**

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	1,769.00
2.	Publication Costs	0.00
3.	Consultant Services	43,316.00
4.	ADP/Computer Services	0.00
5.	Subawards/Consortium/Contractual Costs	0.00
6.	Equipment or Facility Rental/User Fees	0.00
7.	Alterations and Renovations	0.00
8.	Printing/Binding/Postage	1,000.00
9.	Rent/Utilities/Telecommunications	46,500.00
10.	Hospitality	1,500.00
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	94,085.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	596,852.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	375,852.00	30,068.00
		Total Indirect Costs	30,068.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)
DoHS; POC Darryl W. Mayes (214) 767-3261

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	626,920.00

J. Fee

Funds Requested ($)
0.00

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	626,920.00

L. Budget Justification

(Only attach one file.) 2_0_Budget_Justification_5YR_LEND_Comp Add Attachment Delete Attachment View Attachment

RESEARCH & RELATED BUDGET - Budget Period 3

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: M7PNRH24BBM8 **Enter name of Organization:** West Virginia University Research Corporation

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 3 **Start Date:** 07/01/2028 **End Date:** 06/30/2029

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Cassaundra		Miller	EdD	70,720.00	7.80			45,968.00	12,182.00	58,150.00
Project Role: PD/PI											
Dr.	Margaret		Jaynes	MD	228,000.00	0.60			11,400.00	889.00	12,289.00
Project Role: Co-I											
Dr.	Amy		Burt	MD	111,909.00	5.40			50,359.00	13,345.00	63,704.00
Project Role: Clinical Coordinator											

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 134,143.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
2	Secretarial/Clerical	24.00			95,184.00	25,224.00	120,408.00
3	Communications	1.80			8,135.00	2,156.00	10,291.00
1	Data	1.20			7,224.00	1,914.00	9,138.00
2	Clinical Staff	8.64			16,304.00	1,272.00	17,576.00
3	Mentors	5.28			15,404.00	4,082.00	19,486.00
1	Finance	2.40			10,801.00	2,862.00	13,663.00
12	**Total Number Other Personnel**					**Total Other Personnel**	190,562.00

Total Salary, Wages and Fringe Benefits (A+B) 324,705.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	4,323.00
2.	Foreign Travel Costs	
	Total Travel Cost	4,323.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	176,000.00
3.	Travel	
4.	Subsistence	
5.	Other	
18	**Number of Participants/Trainees** — **Total Participant/Trainee Support Costs**	176,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	758.00
2.	Publication Costs	0.00
3.	Consultant Services	43,316.00
4.	ADP/Computer Services	0.00
5.	Subawards/Consortium/Contractual Costs	0.00
6.	Equipment or Facility Rental/User Fees	0.00
7.	Alterations and Renovations	0.00
8.	Printing/Binding/Postage	250.00
9.	Rent/Utilities/Telecommunications	46,000.00
10.	Hospitality	1,500.00
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	91,824.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	596,852.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	375,852.00	30,068.00
		Total Indirect Costs	30,068.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	626,920.00

J. Fee

Funds Requested ($)
0.00

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	626,920.00

L. Budget Justification

(Only attach one file.) 2_0_Budget_Justification_5YR_LEND_Comp

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 4

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: M7PNRH24BBM8 **Enter name of Organization:** West Virginia University Research Corporation

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 4 **Start Date:** 07/01/2029 **End Date:** 06/30/2030

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Cassaundra		Miller	EdD	70,780.00	7.80			46,007.00	12,192.00	58,199.00
Project Role:	PD/PI										
Dr.	Margaret		Jaynes	MD	228,000.00	0.60			11,400.00	889.00	12,289.00
Project Role:	Co-I										
Dr.	Amy		Burt	MD	111,969.00	5.40			50,386.00	13,352.00	63,738.00
Project Role:	Clinical Coordinator										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 134,226.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
2	Secretarial/Clerical	24.00			95,424.00	25,287.00	120,711.00
3	Communications	1.80			8,144.00	2,158.00	10,302.00
1	Data	1.20			7,230.00	1,916.00	9,146.00
2	Clinical Staff	8.64			16,304.00	1,272.00	17,576.00
3	Mentors	5.28			15,443.00	4,092.00	19,535.00
1	Finance	2.40			10,813.00	2,865.00	13,678.00
12	**Total Number Other Personnel**					**Total Other Personnel**	190,948.00

Total Salary, Wages and Fringe Benefits (A+B) 325,174.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	4,323.00
2.	Foreign Travel Costs	
	Total Travel Cost	4,323.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	176,000.00
3.	Travel	
4.	Subsistence	
5.	Other	
18	**Number of Participants/Trainees** — **Total Participant/Trainee Support Costs**	176,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	539.00
2.	Publication Costs	0.00
3.	Consultant Services	43,316.00
4.	ADP/Computer Services	0.00
5.	Subawards/Consortium/Contractual Costs	0.00
6.	Equipment or Facility Rental/User Fees	0.00
7.	Alterations and Renovations	0.00
8.	Printing/Binding/Postage	250.00
9.	Rent/Utilities/Communications	46,000.00
10.	Hospitality	1,250.00
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	91,355.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	596,852.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	375,852.00	30,068.00
		Total Indirect Costs	30,068.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	626,920.00

J. Fee

	Funds Requested ($)
	0.00

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	626,920.00

L. Budget Justification

(Only attach one file.) 2_0_Budget_Justification_5YR_LEND_Comp

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Budget Period 5

OMB Number: 4040-0001
Expiration Date: 01/31/2029

UEI: M7PNRH24BBM8 **Enter name of Organization:** West Virginia University Research Corporation

Budget Type: ☒ Project ☐ Subaward/Consortium **Budget Period:** 5 **Start Date:** 07/01/2030 **End Date:** 06/30/2031

A. Senior/Key Person

Prefix	First	Middle	Last	Suffix	Base Salary ($)	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
Dr.	Cassaundra		Miller	EdD	70,840.00	7.80			46,046.00	12,202.00	58,248.00
Project Role:	PD/PI										
Dr.	Margaret		Jaynes	MD	228,000.00	0.60			11,400.00	889.00	12,289.00
Project Role:	Co-I										
Dr.	Amy		Burt	MD	112,029.00	5.40			50,413.00	13,359.00	63,772.00
Project Role:	Clinical Coordinator										

Additional Senior Key Persons: Add Attachment Delete Attachment View Attachment

Total Funds requested for all Senior Key Persons in the attached file

Total Senior/Key Person 134,309.00

B. Other Personnel

Number of Personnel	Project Role	Months Cal.	Months Acad.	Months Sum.	Requested Salary ($)	Fringe Benefits ($)	Funds Requested ($)
	Post Doctoral Associates						
	Graduate Students						
	Undergraduate Students						
2	Secretarial/Clerical	24.00			95,544.00	25,319.00	120,863.00
3	Communications	1.80			8,153.00	2,161.00	10,314.00
1	Data	1.20			7,236.00	1,918.00	9,154.00
2	Clinical Staff	8.64			16,304.00	1,272.00	17,576.00
3	Mentors	1.80			15,469.00	4,099.00	19,568.00
1	Finance	2.40			10,825.00	2,869.00	13,694.00
12	**Total Number Other Personnel**					**Total Other Personnel**	191,169.00

Total Salary, Wages and Fringe Benefits (A+B) 325,478.00

C. Equipment Description

List items and dollar amount for each item exceeding $5,000

Equipment item	Funds Requested ($)

Additional Equipment: Add Attachment | Delete Attachment | View Attachment

Total funds requested for all equipment listed in the attached file

Total Equipment

D. Travel

		Funds Requested ($)
1.	Domestic Travel Costs (Incl. Canada, Mexico and U.S. Possessions)	4,323.00
2.	Foreign Travel Costs	
	Total Travel Cost	4,323.00

E. Participant/Trainee Support Costs

		Funds Requested ($)
1.	Tuition/Fees/Health Insurance	
2.	Stipends	176,000.00
3.	Travel	
4.	Subsistence	
5.	Other	
18	**Number of Participants/Trainees** — **Total Participant/Trainee Support Costs**	176,000.00

F. Other Direct Costs

		Funds Requested ($)
1.	Materials and Supplies	585.00
2.	Publication Costs	0.00
3.	Consultant Services	43,316.00
4.	ADP/Computer Services	0.00
5.	Subawards/Consortium/Contractual Costs	0.00
6.	Equipment or Facility Rental/User Fees	0.00
7.	Alterations and Renovations	0.00
8.	Printing/Binding/Postage	150.00
9.	Rent/Utilities/Telecommunications	46,000.00
10.	Hospitality	1,000.00
11.		
12.		
13.		
14.		
15.		
16.		
17.		
	Total Other Direct Costs	91,051.00

G. Direct Costs

	Funds Requested ($)
Total Direct Costs (A thru F)	596,852.00

H. Indirect Costs

Indirect Cost Type	Indirect Cost Rate (%)	Indirect Cost Base ($)	Funds Requested ($)
MTDC	8.00	375,852.00	30,068.00
		Total Indirect Costs	30,068.00

Cognizant Federal Agency
(Agency Name, POC Name, and POC Phone Number)

I. Total Direct and Indirect Costs

	Funds Requested ($)
Total Direct and Indirect Institutional Costs (G + H)	626,920.00

J. Fee

Funds Requested ($)
0.00

K. Total Costs and Fee

	Funds Requested ($)
Total Costs and Fee (I + J)	626,920.00

L. Budget Justification

(Only attach one file.) 2_0_Budget_Justification_5YR_LEND_Comp

Add Attachment | Delete Attachment | View Attachment

RESEARCH & RELATED BUDGET - Cumulative Budget

		Totals ($)
Section A, Senior/Key Person		660,955.00
Section B, Other Personnel		902,781.00
Total Number Other Personnel	60	
Total Salary, Wages and Fringe Benefits (A+B)		1,563,736.00
Section C, Equipment		
Section D, Travel		20,615.00
1. Domestic	20,615.00	
2. Foreign	0.00	
Section E, Participant/Trainee Support Costs		902,000.00
1. Tuition/Fees/Health Insurance		
2. Stipends	902,000.00	
3. Travel		
4. Subsistence		
5. Other		
6. Number of Participants/Trainees	90	
Section F, Other Direct Costs		497,909.00
1. Materials and Supplies	15,429.00	
2. Publication Costs	0.00	
3. Consultant Services	239,580.00	
4. ADP/Computer Services	0.00	
5. Subawards/Consortium/Contractual Costs	0.00	
6. Equipment or Facility Rental/User Fees	0.00	
7. Alterations and Renovations	0.00	
8. Other 1	4,150.00	
9. Other 2	231,500.00	
10. Other 3	7,250.00	
11. Other 4		
12. Other 5		
13. Other 6		
14. Other 7		
15. Other 8		
16. Other 9		
17. Other 10		

Section G, Direct Costs (A thru F)	2,984,260.00
Section H, Indirect Costs	148,580.00
Section I, Total Direct and Indirect Costs (G + H)	3,132,840.00
Section J, Fee	0.00
Section K, Total Costs and Fee (I + J)	3,132,840.00

DISCLOSURE OF LOBBYING ACTIVITIES

Complete this form to disclose lobbying activities pursuant to 31 U.S.C.1352

OMB Number: 4040-0013
Expiration Date: 06/30/2028

1. * Type of Federal Action:

- [ ] a. contract
- [x] b. grant
- [ ] c. cooperative agreement
- [ ] d. loan
- [ ] e. loan guarantee
- [ ] f. loan insurance

2. * Status of Federal Action:

- [x] a. bid/offer/application
- [ ] b. initial award
- [ ] c. post-award

3. * Report Type:

- [x] a. initial filing
- [ ] b. material change

4. Name and Address of Reporting Entity:

[x] Prime [ ] SubAwardee

** Name* West Virginia University Research Corporation

** Street 1* 886 Chestnut Ridge Road *Street 2*

** City* Morgantown *State* WV: West Virginia *Zip* 26506-6845

Congressional District, if known: WV-002

5. If Reporting Entity in No.4 is Subawardee, Enter Name and Address of Prime:

6. * Federal Department/Agency:

HRSA

7. * Federal Program Name/Description:

Autism Collaboration, Accountability, Research, Education, and Support

Assistance Listing Number, *if applicable:* 93.877

8. Federal Action Number, *if known:*

9. Award Amount, *if known:*

$

10. a. Name and Address of Lobbying Registrant:

Prefix ** First Name* N/A *Middle Name*

** Last Name* N/A *Suffix*

** Street 1* *Street 2*

** City* *State* *Zip*

b. Individual Performing Services (including address if different from No. 10a)

Prefix ** First Name* N/A *Middle Name*

** Last Name* N/A *Suffix*

** Street 1* *Street 2*

** City* *State* *Zip*

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*** Signature:** Cathy.Tarabrella

***Name:** *Prefix* ** First Name* Cathy *Middle Name*

** Last Name* Tarabrella *Suffix*

Title: Associate Director of OSP, Pre-Award **Telephone No.:** 304-293-3998 **Date:** 07/10/2026

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RESEARCH & RELATED Other Project Information

OMB Number: 4040-0001
Expiration Date: 01/31/2029

1. Are Human Subjects Involved? ☐ Yes ☒ No

1.a. If YES to Human Subjects

Is the Project Exempt from Federal regulations? ☐ Yes ☐ No

If yes, check appropriate exemption number. ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5 ☐ 6 ☐ 7 ☐ 8

If no, is the IRB review Pending? ☐ Yes ☐ No

IRB Approval Date:

Human Subject Assurance Number:

2. Are Vertebrate Animals Used? ☐ Yes ☒ No

2.a. If YES to Vertebrate Animals

Is the IACUC review Pending? ☐ Yes ☐ No

IACUC Approval Date:

Animal Welfare Assurance Number:

3. Is proprietary/privileged information included in the application? ☐ Yes ☒ No

4.a. Does this Project Have an Actual or Potential Impact - positive or negative - on the environment? ☐ Yes ☒ No

4.b. If yes, please explain:

4.c. If this project has an actual or potential impact on the environment, has an exemption been authorized or an environmental assessment (EA) or environmental impact statement (EIS) been performed? ☐ Yes ☐ No

4.d. If yes, please explain:

5. Is the research performance site designated, or eligible to be designated, as a historic place? ☐ Yes ☒ No

5.a. If yes, please explain:

6. Does this project involve activities outside of the United States or partnerships with international collaborators? ☐ Yes ☒ No

6.a. If yes, identify countries:

6.b. Optional Explanation:

7. Project Summary/Abstract Project_AbstractSummary_2_0_V2_0__1_. Add Attachment Delete Attachment View Attachment

8. Project Narrative LEND_Narrative.pdf Add Attachment Delete Attachment View Attachment

9. Bibliography & References Cited Add Attachment Delete Attachment View Attachment

10. Facilities & Other Resources Add Attachment Delete Attachment View Attachment

11. Equipment Add Attachment Delete Attachment View Attachment

12. Other Attachments Add Attachments Delete Attachments View Attachments

INTRODUCTION

The goal of the Maternal and Child Health Leadership Program, entitled Leadership Education in Neurodevelopmental and Other Related Disabilities (LEND), and specifically WV (WV) LEND is to **train students, practicing professionals, and individuals with disabilities, and families members of people with disabilities** to be leaders in disability education, research, and the screening, diagnosis, and provision of services for children and youth with autism/DD across the lifespan by:

1. Providing evidence-based interdisciplinary education and training that advances the understanding of children and youth with autism/DD and their families, by increasing academic and clinical knowledge, and building leadership skills for trainees including students, individuals with lived disability experience, and practicing professionals.
2. Increasing the identification and use of evidence-based screening, diagnostics, and treatment for children and youth with autism/DD by providing exemplary and high quality, family-centered, and interdisciplinary collaborative academic and experiential training opportunities.
3. Developing and enhancing innovative practice models that strengthen systems of care for individuals with autism/DD across the lifespan in rural communities built through partnerships among disciplines across WV University (WVU), WVU Medicine and other state medical systems of care, community, state, regional, and national entities.
4. Increasing the expertise of trainees and practicing professionals from local and state agencies, community organizations, and private providers to recognize the complex lifespan needs of children and youth with autism/DD with focus on the lived experience perspective to improve rural systems of care.

Building on 25 years of Maternal and Child Health (MCH) leadership and training experience, WV LEND prepares the next generation of interdisciplinary disability leaders grounded in the Maternal Child Health (MCH) principles of family-centered, systems-integrated, and lifespan-focused care for children and youth with autism/DD and their families. WV LEND equips trainees to ensure that children and youth with autism/DD receive evidence-based screening, evaluation, diagnosis, intervention and ongoing supports across the lifespan.

WV LEND expands the capacity of the interdisciplinary workforce to meet the complex needs of children and youth with autism/DD and their families throughout WV's predominantly rural communities through the development of new partnerships and the strengthening of longstanding collaborations. These relationships across the WVU system, state agencies, community organizations, family partners, regional organizations, and national networks enhances leadership development, strengthens systems of care, and improves access to high-quality, evidence-based services.

WV LEND also partners with three other LEND programs through the LEND Rural Collaborative, creating opportunities to share expertise, develop and disseminate rural-specific training, expand interdisciplinary and clinical knowledge, and cultivate leadership focused on improving and optimizing outcomes for children and youth with autism/DD across their lifespan and their families living in rural communities.

Housed within and closely integrated into the WV University Center for Excellence in Disabilities (WVUCED), WV LEND builds upon an established infrastructure, interdisciplinary expertise, and statewide partnerships to advance leadership development, workforce capacity, systems improvement, and sustainable change. By combining 25 years of MCH experience with innovative partnerships and emerging models of care, WV LEND is positioned to prepare healthcare leaders capable of navigating and transforming systems of care and improving the health and well-being of children and youth with autism/DD and their families throughout WV and the broader rural Appalachian region.

NEED

WV LEND Needs Assessment Outcomes: To inform future WV LEND training objectives, the WV LEND developed and distributed a needs assessment to a diverse group of state stakeholders engaged in disability services, family supports and education. Input from individuals with lived experience, community members, mentors, and faculty ensured the assessment reflected the experiences and priorities of those who provide services, receive services, and support disability initiatives across communities. The survey asked participants to rate the importance of competencies drawn from the MCH Leadership Competencies (Version 5), including maternal and child health, self-reflection, ethics, problem solving, policy development, and more. Results guided curriculum development to ensure training activities align with identified priorities and evolving workforce needs.

Fifteen stakeholders completed the survey representing professionals, mentors, community members, individuals with lived disability experience and other disability leaders. Overall, participants rated nearly all proposed leadership competencies as either very important or extremely important, demonstrating broad consensus on the need for comprehensive training for future disability leaders. The highest rated competencies included **working through problems, using a team approach**, and **considering individuals, environments, and available resources when building services and supports**, with every respondent rating these areas as either very important or extremely important.

Open-ended responses reinforced the importance of training that extends beyond foundational leadership skills. Participants emphasized the need for content related to person-centered and trauma-informed care, evidence-based practices, community skills, community involvement, resource navigation, accessible language, and more. Collectively, these findings support the development of an interdisciplinary leadership curriculum that combines technical and clinical

knowledge with improving systems of care, collaboration, and person/family-centered competencies to prepare future disability leaders.

WV Identified Needs: The complex social and economic characteristics of WV and the surrounding Appalachian region present multiple risks to the health and related needs of children and families, and are even more pronounced for the target population of LEND – children and youth with autism/DD across their lifespan and their caregivers/families. Geographic, social, and economic isolation are defining characteristics of WV's predominately rural population. Almost 50% of WVns live in rural areas, more than doubling the national average of 20% (United Health Foundation). As the second most rural state and the only state located entirely within Appalachia, WV is disproportionately affected by chronic economic and social distress.

The geographical environment of WV contributes to widespread community health factors including poverty, food insecurity, inadequate housing conditions, limited or no access to safe drinking water, higher than the national average unemployment, and under-resourced health care and social systems. One in three West Virginians earn below 150% of the federal poverty level (APPLI, 2026) and 25% of WV families live in food deserts with limited or no access to affordable, nutritious food compared to a 5.6% national average (USDA, 2026).

Finances and travel are particularly burdensome given the socioeconomic realities facing many WV families. More than one in five children with special health care needs live below the federal poverty level (O'Leary, 2025). As one of the nation's most rural states, WV also has limited interstate highway access, mountainous terrain, and aging transportation infrastructure- with 60% of roads rated as fair to poor. Consequently, many families must travel considerable distances and rely on often limited resources to obtain developmental services, creating financial and logistical hardships that can delay evaluation, diagnosis, and intervention.

Because of intersecting health and social gaps, WV children and families also experience substantial barriers to optimal developmental and health outcomes, especially for children and youth with autism/DD. Nationally, 1 in 31 children have been diagnosed with autism (CDC, 2026). Estimates in WV indicate similar or a slightly higher prevalence along with a consistently elevated identification of DD. To illustrate, the 2022-2023 National Survey for Child Health Data found 26.2% of children and youth in the United States have special health care needs compared to WV's rate of 33.8%. WV has the second highest percentage of children and youth with special health care needs in the nation. WV's rate of children with special health care needs rose almost 10% since the 2017-2018 National Survey for Child Health Data was completed. Notably, across the lifespan, 37% of adults in WV report having a disability with the national average reported as 28.7% (WVUCED, 2026). The number of West Virginians impacted by autism/DD highlights an increasing need for coordinated, multidisciplinary systems of care led

by highly trained providers informed by the lived experiences of individuals with disabilities and their families.

Appalachia has a limited workforce trained to meet the complex needs of children and youth with autism/DD and their families. WV has a critically limited number of developmental pediatricians, with approximately two providers identified statewide through publicly available directories. The WV LEND Medicine/Pediatric Provider Mentor is one of the two developmental physicians and also serves as the Medical Director of the WVU Medicine Golisano Childrens' Neurodevelopmental Center (NDC). The national average of developmental pediatricians is 1:100,000 children (Baum et al., 2024) with WV, the approximate ratio is 0.3:100,00. With this shortage of developmental providers, WV children and youth often experience lengthy wait times exceeding six months- sometimes extending past two years. Families may rely on non-speciality providers for developmental assessment and clinical care coordination or seek care from out-of-state specialists. Most often, referrals to out-of-state providers are not covered by WV state insurance so these services are limited by insurance restrictions, transportation challenges, and out-of-pocket costs.

WV LEND maintains a strong partnership with the (NDC), WV's primary center for autism/DD interdisciplinary evaluation and treatment. Through this partnership, trainees receive mentorship from the NDC's developmental pediatrician through direct instruction and clinical guidance in evidence-based screening, evaluation, diagnosis, and intervention for autism/DD. This unique training opportunity provides trainees exposure to expert providers in the field of autism/DD and best practice standards in interdisciplinary care all while building capacity to directly address critical workforce shortages in the state.

Unfortunately, a delay in evaluation and diagnosis can also delay interventions such as speech therapy, occupational therapy, and behavioral and mental health supports. In addition, not all rural providers are well equipped to provide services to children and youth with autism/DD. Lindsay et al. (2026) identified the need for more training in disability, access to ongoing professional development, and recruitment and retention of providers as major impediments to improving the system of care for children and youth with disabilities who live in rural communities. Bican et al. (2025) expanded on rural disability care by examining the gaps in care for children with DD living in rural and low-resourced communities. Parents involved in the study reported a lack of local specialists, long waitlists, and large travel distances to clinics as factors that prevent timely and competent care for their children with autism/DD. The average drivetime to specialized care was 1.5 hours. Over 60% of the parents in this study reported a delay in diagnosis for their child because of waitlists. On average, parents identified a delay in their child's development at 2.3 years old but did not receive a diagnosis until 4.3 years old.

To help address the provider shortage in WV with expertise in serving children with autism/DD, WV LEND trainees participate in the WVUCED Feeding and Swallowing Clinic. This hands-on, multidisciplinary clinic provides comprehensive developmental evaluations, feeding assessments, and resource needs assessment for children with complex feeding challenges. WV LEND trainees train alongside Lived Experience Mentors (family and individual with disabilities) as well as mentors from speech therapy, occupational therapy, developmental pediatrics, nutrition, and social work. These mentors are recognized leaders in both their disciplines and in the field of disability services. Feeding difficulties are extremely prevalent in this population, with studies identifying up to 89% of children with autism/DD experiencing difficulty with eating (Gent et al., 2026). The WVUCED Feeding and Swallowing Clinic is the only multidisciplinary assessment clinic in WV. Families travel from across the state to receive specialized evaluation and intervention services. WV LEND, recognizing the difficulties with travel, offers telehealth as a means for seeing children and youth with their families. Through participation in in-person and virtual clinics, WV LEND trainees gain unique interdisciplinary clinical experience that would otherwise be unavailable to many future providers in WV.

For individuals living in Appalachia, medical mistrust is a longstanding hurdle stemming from a history of shortages, closures, and lack of clear communication. In a 2023 study (Thomas et al.), over 25% of respondents living in Appalachia reported high levels of health care mistrust. This lack of confidence in the medical system can delay care and promote hesitancy in seeking support from outside agencies and supports thus decreasing health outcomes. In 2021 Efird et al. identified patient-centered practices, improved systems of care, and consistency of services as methods to increase trust in health care within Appalachian communities. The findings of this study align with WV LEND training objectives including family-centered care, interdisciplinary collaboration, shared decision making and relationship-building with children and youth with autism/DD and their families.

Provision of services to rural populations requires an understanding of the distinct learning and communication patterns with Appalachian communities. Recent studies confirm the persistence of lower literacy and lower educational attainment among individuals living in Appalachia (APPLI, 2026). In WV, three in five adults exhibit reading proficiency below an 8th level, and one in five read below a 3rd grade level. These gaps in learning illustrate a need for multimodal information sharing in health care including the use of plain language, a team approach, and improved systems of care to support children and youth with autism/DD and their families to achieve optimal outcomes. Within WV LEND, trainees are trained to create accessible documents that meet federal accessibility standards, utilize plain language, and deliver clear, family-centered communication that increases understanding. In clinics mentors, including the lived experience mentors, and trainees provide feedback on effective communication.

Children and youth with autism/DD across their lifespan and their families in WV face multiple hurdles to receiving screening, evaluation, diagnosis and intervention that promotes optimal developmental outcomes. Again, 33.8% of children in WV require specialized care, compared to 26.2% nationally (National Survey for Child Health Data, 2022-2023). WV families face high poverty rates, rural isolation, food insecurity, limited transportation infrastructure, and a critically low number of developmental specialists. For children and youth with autism/DD and their families, living in WV often results in extended waitlists for evaluation and services, limited coordination of services, limited to no interdisciplinary collaboration, and lengthy travel to access care. WV LEND is uniquely positioned to **educate existing providers, recruit and train new providers, engage trainees through a variety of training opportunities, increase the number of diagnostic evaluations completed by LEND mentors/faculty, and provide technical assistance to existing agencies to improve systems of care through the lens of lived disability experience and professional clinical leadership**.

Approach

Overall Methodology

This section outlines the goals and measurable outcome-based objectives which will be accomplished by WV LEND within the stated timeframe (Attachment 1, Work Plan). High quality trainees with leadership potential will be trained utilizing academic and experiential components of the described curriculum allowing them to master the competencies that will prepare them to succeed in the workforce by meeting the complex needs of children and youth with autism/DD.

Yearly, WV LEND will develop at least three continuing education activities to engage practicing professionals within the WV LEND framework. This section describes the content of the training program as it relates to fundamental MCH philosophy. Specific training activities will be geared to clinical and non-clinical trainees occurring in a variety of settings and at varying levels of intensity and will be described by purpose, methodology, content, time commitment and method of evaluation (Attachment 1 Work Plan; Attachment 5 Curriculum). WV LEND builds on past experiences and outcomes developing trainees prepared to be leaders in the workforce providing services to individuals with autism/DD.

Dissemination of WV LEND reports, products, research projects, and other project outputs occur through established collaborations with the WVUCED, AUCD, WVU, and other community partners/agencies. The WVUCED maintains an extensive dissemination network and dedicated communications team, reaching approximately 5,000 individuals with disabilities, individuals with lived disability experience, policymakers, stakeholders, educators, current/past LEND trainees, and community organizations- many local to WV. The WVUCED works closely with both the AUCD and WVU to widen the WV LEND reach to state and national audiences. WVU has a statewide reach through clinical, educational, and community engagement networks, ensuring information and products from WV LEND reach rural communities across WV and

beyond. The AUCD further amplifies dissemination efforts through national communication platforms, conferences, webinars, publications, and professional networks extending the reach of WV LEND products, training, and resources to a broad national audience highly engaged in improving the lives of individuals with autism/DD.

WV LEND, as part of the WVUCED, funds 7% FTE of the Dissemination Officer of the WVUCED Communications Team to coordinate communication, assist with development of products, and disseminate information related to the program and available services and training. The Dissemination Officer meets yearly with the Program Director, Co-Program Director, Training Director, Data Analyst and Program Manager to develop a WV LEND dissemination plan to help identify data to be targeted and to translate data to demonstrate impact.

Goals and Objectives

Goal 1: *To provide evidence-based interdisciplinary education and training that advances the understanding of children and youth with autism/DD across their life span and their families by increasing academic and clinical knowledge and building leadership skills for trainees and mentors including students, individuals with lived disability experience, and practicing professionals.*

Objective 1-1: Identify, recruit, select and retain a minimum of 18 long-term trainees from 10 disciplines per year.

Objective 1-2: Identify, recruit and retain 15 medium-term trainees per year.

Objective 1-3: Identify, recruit and retain mentors from 9 different disciplines per year with at least one lived experience and one family mentor.

Objective 1-4: Provide disciplinary and interdisciplinary experiential and academic leadership training to 18 long-term and 15 medium-term trainees per year.

Objective 1-5: Identify, develop and provide, each year, three continuing education (CE) opportunities through conferences, workshops or similar training opportunities for practicing professionals serving children, youth, and adults with autism/DD in the state and region.

Objective 1-6: Increase trainees' knowledge of MCH concepts, such as medical home, shared decision making, leadership, systems of care, and interdisciplinary care as they relate to individuals across the lifespan with autism/DD in rural communities.

Objective 1-7: Provide academic and experiential forums so trainees recognize and value seamless and accessible systems of care specific to the perspectives of lived experience of individuals across the lifespan with autism/DD and their families/caregivers.

Objective 1-8: Empower trainees to utilize leadership skills within organizations to increase broader systems thinking to include lived experience perspective and science driven initiatives related to autism/DD.

Objective 1-9: Understand and discuss MCH population focused legislation that addresses health care delivery systems for individuals with autism/DD and their families/caregivers.

Objective 1-10: Increase by 10% yearly the number of long-term trainees, reporting on trainee surveys, engagement in policy development, implementation and evaluation.

Goal 2: *To increase the identification and use of evidence-based screening, diagnostics, and treatment for children and youth with autism/DD across their lifespan by providing exemplary and high quality, family-centered, and interdisciplinary collaborative academic and experiential training opportunities.*

Objective 2-1: Provide 18 long-term and 15 medium-term trainees, through curriculum forums, with the conceptual knowledge of the characteristics and complexities of autism/DD.

Objective 2-2: Provide LEND trainees four observations of interdisciplinary screening, evaluation, and diagnosis for autism/DD.

Objective 2-3: Increase the number of practicing professionals skilled in the differential diagnosis of autism/DD in primary care/community setting by providing both large group and individualized technical assistance and continuing education.

Objective 2-4: Provide trainees, through academic and experiential forums, the conceptual knowledge of evidence-based intervention for autism/DD.

Objective 2-5: Provide trainees with four interdisciplinary experiences emphasizing high quality, family-centered and community care based on the educational and environmental needs of children, youth and adults newly diagnosed with autism/DD.

Objective 2-6: Provide trainees with four interactive experiences demonstrating evidence-based intervention for autism/DD.

Objective 2-7: Increase the capacity of rural systems of care to utilize innovative practice models, such as telehealth, to expand diagnostic services for individuals with autism/DD through five technical assistance through community outreach activities a year.

GOAL 3: *To develop and enhance innovative practice models that strengthen systems of care for individuals with autism/DD across their lifespan in rural communities built through partnerships among disciplines across WV University (WVU), WVU Medicine and other state medical systems of care, community, state, regional, and national entities.*

Objective 3-1: Provide trainees, through the use of curriculum forums, with the conceptual knowledge and skills necessary to lead family-centered interdisciplinary processes in a variety of rural community settings.

Objective 3-2: Ensure trainees demonstrate competency for varying perspectives in tandem with leadership skills in clinical and community-based settings that reflect an understanding of rural community health factors.

Objective 3-3: Develop or enhance partnerships across WVU Medicine system, the WVU academic system, State Title V agencies, other MCH or MCH-related programs and regional/national LENDs to demonstrate evidenced-based practice to promote solutions to the needs of children, youth and adults with autism/DD.

Objective 3-4: Provide trainees and mentors with five opportunities to collaborate with State Title V agencies, other MCH or MCH related programs through service, training, continuing education, technical assistance, product development and research.

Objective 3-5: Provide trainees and mentors with four opportunities to engage in innovative service models to improve the well-being of children and youth with autism/DD and their families in rural communities.

Objective 3-6: Provide, with both trainees and mentors, consultation, technical assistance, dissemination of information and continuing education activities for at least 200 practicing professionals in the state, region, and nation each year.

Goal 4: *Increase the expertise of trainees and practicing professionals from local and state agencies, community organizations, and private providers to recognize the complex lifespan needs of children and youth with autism/DD across their lifespan with focus on the lived experience perspective to improve rural systems of care.*

Objective 4-1: Provide trainees, through the use of curriculum and experiential forums, with the conceptual knowledge and performance skills necessary to participate in and contribute to the interdisciplinary team process, including medical home, community care, school involvement and follow-ups in rural systems of care for children and youth with autism/DD and their families.

Objective 4-2: Support trainees to plan, design, and present research projects/badge projects through mentor/faculty and trainee collaboration and partnerships with practicing professionals focused on the lifespan care of individuals with autism/DD.

Objective 4-3: Provide opportunities for trainees to demonstrate the ability to translate evidence-based research into practice by linking academic and experiential forums.

Objective 4-4: Provide trainees with opportunities for participation in four innovative model service delivery experiences (e.g., LEND Support Program, Feeding and Swallowing Clinic,

Nutrition Services, Telehealth Clinics, and Assistive Technology Clinic) that demonstrate family-centered interdisciplinary approaches to care.

Objective 4-5: Prepare individuals with autism/DD and their families/caregivers to improve systems of care by using an experiential model to demonstrate self-directed care coordination, shared decision making, and family-centered approaches.

Objective 4-6: Increase by 10% per year the percent of long-term trainees who at 5 and 10 years post training, work in an interdisciplinary manner to serve the MCH population and, in particular, individuals with autism/DD and their families.

The Work Plan which describes the objectives by method, person(s) responsibilities, evaluation and timeline is located in Attachment 1.

Trainee Recruitment and Retention

WV LEND is applying for continuation of its current Tier 3 Level. WV LEND recruits high quality trainees who demonstrate leadership potential and express an interest in working with children and youth with autism/DD across their lifespan and those who care for them. Potential trainees are recruited from a variety of sources including advertisements in the student newspaper, announcements in Disability Studies Minor or Certificate courses, the WVUCED website, WVUCED/WVU/AUCD social media postings, emails from their respective disciplines and from discipline mentors. Many of our trainee applicants report first hearing about LEND from a current trainee. Past and current trainees refer and recruit multiple trainee applicants each year. WV LEND staff also attend university, community, and statewide student recruitment fairs and events to create larger awareness and increased interest in WV LEND. Other trainee applicants may have observed WVUCED interdisciplinary clinics at the suggestion of their discipline mentors, attended a WVUCED or WV LEND training or conference, or completed a project or academic placement/fieldwork at the WVUCED or with a WV LEND mentor. For instance, student volunteers who have participated in the WVU Neurodevelopmental Center Autism Clinic, which provides intensive, individualized one-to-one intervention to children with autism/DD, have gone on to become LEND trainees.

WV LEND strives to recruit trainees from variety of disciplines and has fostered and maintained collaborative relationships with many WVU academic programs. For example, disciplines such as the Speech-Language Pathology Master's program include WV LEND trainee opportunities within their recruitment of incoming students. The Physician Assistant Studies Program at WVU promotes LEND for their students and provides special recognition for WV LEND trainees during their graduation awards ceremony. The WVU School of Dentistry and WVU Department of Occupational Therapy recommend graduate students consider WV LEND. WV LEND includes trainees from other WV higher education institutions including Marshall University.

Lived disability experience trainees, including individuals living with a disability and families of individuals with disabilities, are recruited using the WVUCED's wide network of outreach to the disability community including the Community Advisory Council, WV Developmental Disability Council, local/regional/statewide support groups, email list serve, and recruitment announcements at conferences, health fairs, and other community events. WVUCED also connects with AUCD social media to promote WV LEND recruitment.

Applicants are required to complete an application asking them to reflect on their leadership qualities, skills and attributes they hope to develop by being a trainee. To ensure interest and commitment to serving children and youth with autism/DD across their lifespan and their families, the WV LEND application includes describing prior disability experience, professional goals and interest in autism/DD. Applicants are interviewed and chosen by a team of WV LEND core faculty, including at least one mentor with lived disability experience, and at least one current trainee. A strong interest and verbal commitment to want to serve individuals with autism/DD is a requirement for all selected WV LEND trainees.

All application and interview materials are made available to each applicant based on their individualized accommodation needs. All WV LEND/WVUCED meet or exceed federal accessibility requirements. WV LEND collaborates with WVUCED's communication/media team and WV Assistive Technology System (WVATS) to ensure full access to all interested applicants.

Long-term trainees include master's and doctoral candidates, selected from LEND approved disciplines or who intend to provide autism/DD services upon entering the workforce. In addition, at least one individual with a disability and one family member of an individual with a disability will be included in each trainee cohort. In 2025-26, twelve of the nineteen trainees identified as having a disability and/or as being a family member of an individual with a disability. Community trainees with at least a master's degree who serve individuals with autism/DD also be considered. No single discipline makes up more than 25% of a yearly cohort.

For 2025-2026, trainees were chosen from over 60 applicants from 17 disciplines. Within the 2025-2026 cohort, trainees included, an undergraduate student with a physical disability who runs a comedy podcast, the sister of an individual with autism, a social work graduate student who is also the director of a community autism center, a third year law trainee who also has a PhD in education and will be pursuing a career in advocacy law practice, a practicing developmental specialist from WV Birth to Three (Title V early intervention), and graduate students from physical, occupational, and speech therapy. WV LEND received over 100 applications for consideration for the 2026-2027 cohort.

All trainees are paired with primary mentors from the same or similar disciplines. These mentors are recognized leaders in their respective academic, clinical, professional or lived experience

areas and have demonstrated longstanding commitments to improving the lives of children and youth with autism/DD and their families.

Including mentors with disabilities and mentors who are family members of individuals with disabilities has strengthened the family-centered focus of WV LEND. WV LEND is committed to developing a sustainable pipeline of individuals with disabilities and family members of individuals with disabilities who will apply their new leadership skills in future roles as advisory board members, mentors, and community leaders. Three LEND trainees have gone on to become LEND lived experience mentors. In addition, WVUCED parent and family-led programs, including the Paths for Parents Program and the Family to Family Program, provide ongoing opportunities for trainees with lived experience to continue their engagement through leadership, mentoring, and family support activities.

Academic trainee support is provided in accordance with policies mandated by the WVU Office of Graduate Education and Life and academic departments. Trainees are funded for 9 to 12 months with summer activity hours counting towards their 300+ hour requirement.

Each trainee, in consultation with their primary mentor and the Training Director, will develop an Individual Training Plan (ITP) which consists of three goals and corresponding action steps to support goal attainment. Each trainee completes the MCH Leadership Self-Assessment which is used to identify their ITP leadership goal. The second goal is a research/policy goal and the third goal is a personal development goal. Through the first semester each trainee will identify a project, as part of the Disability Micro-Credential Badge, that may become the foundation for research. Badge projects could be related to work within the trainee's discipline or other activities from within the CED programs. All activities and level of mentor support will be modified as appropriate based on the needs of the trainee. Trainees may include additional mentors in the development of their ITP, on projects, or for mentorship at any time. Most trainees have a working relationship with multiple mentors throughout their time with WV LEND. Importantly, all trainees receive mentorship from the program mentor with a disability and the family mentor.

WV LEND provides a dedicated trainee room for all trainees. The trainee room includes computers with high speed internet, printing, a lending library of disability-related books, a space to collaborate with fellow cohort-mates or work independently, and presentation technology.

Medium-term trainees are recruited for WV LEND programming from a variety of sources. WVU pediatric residents and child psychiatry fellows will be recruited during their child development rotation. Speech-language pathology, social work and occupational therapy practicum students are also offered participation in WV LEND as a medium-term trainee. Other students, families, and individuals with disabilities often become medium-term trainees after seeking additional learning from WV LEND or after engaging in a WVUCED or other LEND

activity. Trainees who interviewed, but were not selected, for WV LEND are also recruited as medium-term trainees.

Each trainee, under the mentorship of WV LEND faculty, will complete 40+ hours of defined curriculum that reflects the LEND curriculum and training content. This curriculum will include at least 19 clinical hours and 22 didactic hours to enhance the trainee's knowledge and skills. (Attachment 5: Syllabi for Curriculum Forums). Medium-term trainees will participate in core activities, interdisciplinary clinics and LEND Journal Club alongside current long-term trainees. Medium-term trainees will complete online LEND learning modules built as a Disability Micro-Credential Badge. Students who completed the Disability Micro-Credential Badge can also be considered for medium-term trainee status. Badge learning topics include: family/person-centered care, lived disability experience, autism/DD across the lifespan, evidence-based practices in evaluation and intervention for children and youth with autism/DD across the lifespan, building universal documents and using plain language, and leadership. The Disability Identity Badge is included in the Mountaineer Micro Academy which, upon completion, allows a connection to LinkedIn® and is certified through the WVU Provost Office. Each medium-term trainee identifies optional activities to complete as part of their training.

All medium-term trainees have access to the trainee room and mentor support. Medium-term trainees also participate in training, clinics, and webinars alongside long-term trainees.

WV LEND program will increase medium-term trainees numbers by 10% each year and include new disciplines. For example, WVU medical students between their first and second year can participate in a two week program of interest so WV LEND will advertise and recruit for the Disability Micro-Credential Badge for students interested in such areas as disability, child development, neurology, physical medicine and rehabilitation. Students will be offered a letter for their files indicating their completion of the Disability Badge.

All **former trainees** are contacted at 5 and 10-year intervals. During their exit interviews trainees provide a permanent email and mailing address to be contacted to complete follow-up surveys. Survey format and importance are reviewed with trainees during exit interviews and throughout their training with WV LEND. Trainees also receive phone calls to gain follow-up information. Former trainee "friends" through Facebook/Instagram will receive requests encouraging them to share information including employment, research, programs initiated, publications, community engagement, and other leadership activities. Web-based texting results in the largest amount of engagement from past trainees. A secure repository of current and past trainee phone numbers allows continued contact with trainees. Also, former trainees with known email addresses are placed on a list-serve which will allow WV LEND to remain engaged with trainees throughout the years. Upcoming WV LEND activities and opportunities for engagement along with intermittent program updates will be shared on social media platforms and via email/text. Mentors and other WV LEND staff will serve as points of contact for former trainees.

Training and Education

The educational philosophy guiding the **curriculum and education content** of the WV LEND training program is based on preparing a wide range of trainees including family members, individuals with disabilities, practicing professionals, and graduate students with advanced disability training to prepare for new leadership roles within the disability community and emerging healthcare systems for special healthcare needs, including autism/DD. WV LEND curriculum framework is built from four foundational pillars: academic, clinical, community, and leadership. The content of this four-pillared instructional program is presented by a variety of methods across multiple forums in order to engage all trainees as adult learners and is reflective of the goals of the WV LEND Program. Considering the trainees as learners increases their ability to self-direct their learning, enabling them to be ready to continue learning as they move through their life and career paths. The WV LEND Program utilizes both conceptual and performance training strategies to develop sustained skills and critical thinking to prepare trainees to be higher performers and leaders in healthcare and disability community settings.

The following identifies the curriculum and education content of the WV LEND Program ensuring trainees gain understanding and experiences necessary to prepare them for leadership positions in a changing healthcare environment while promoting the values of MCH. The content and activities utilized to reach the goals of the program are based on MCH training program and MCH leadership goals, including: training in developmental disabilities and other related disabilities, with the focus on autism; leadership skills; interdisciplinary skills; public health perspective and systems of care for those with disabilities; cultural and linguistic responsiveness; quality improvement; family-centered care; best practices and emerging issues in the field of autism/DD; transition; and research and program evaluation. These are **highlighted** below as they are described. Do note, the activities are described as separate tasks. However, they are not independent of one another as trainees. For example, a trainee will reflect on how an encounter with a family might be impacted by the findings from a journal club article. These experiences allow trainees to identify and transform their leadership skills, in a developmental progression, so that they impact others and, eventually, the wider community and its policy. The community partnerships serve to enhance the experiences for the trainees and serve as a collaborative effort to build capacity throughout the state, especially for autism supports.

LEND trainees participate in a **core curriculum** presented through a myriad of **academic and experiential** forums in order to meet the competencies to improve services for individuals with autism/DD. The WV LEND curriculum includes **ten forums**: Introductory Disability Institute, Academic Content, Family Engagement, Leadership Seminar, Clinical, Community, Disability Identity Badge and Project, Research, Journal Club, and Self Reflection. Interdisciplinary care is present “in everything we do”.

Mentorship is also a large part of WV LEND. Mentors frequently meet with trainees to support skill development, reflect and evaluate performance, and continually foster professional growth and leadership development. Each trainee is assigned a primary mentor matched with their trainee discipline, and a secondary mentor selected based on an area of interest within LEND such as clinical practice, leadership, policy, or lived experience. Trainees also receive mentorship from various other LEND faculty and mentors, clinical partners, community professionals, and self-advocates throughout their training experiences. Mentorship is the foundation upon which trainees develop their confidence, competence, and collaborative leadership skills to improve systems of care and positively impact the lives of individuals with autism/DD and their families.

The Curriculum and Education content is summarized in **Attachment 5** which includes an **overview of the LEND Curriculum**, **Syllabi for Curriculum Forums** including the **Disability Immersion program for Medium-Term trainees**, **Trainee Leadership Competencies by Curriculum Forums and Practicum Sites** with accompanying learning objectives.

Interdisciplinary training is exemplified in many WV LEND activities. For example, trainees will observe and participate in the WVUCED Feeding and Swallowing Clinic **chart reviews, evaluations, and follow-up visits**. Mentor disciplines included during these sessions are: speech language pathology, occupational therapy, nutrition, developmental pediatrics, social work, education, family, and mentor with a disability. Other focused interdisciplinary interactions occur during leadership seminars when mentors, with the seminar leader, discuss current and emerging topics related to disabilities or leadership concepts alongside the trainees. Multiple perspectives and experiences are shared during the journal clubs through interdisciplinary discussions with mentors and trainees, not only from WVU but from other rural LEND programs across the United States.

Successful trainees will have a variety of experiences to help them understand and appreciate the needs of infants, children and adolescents who have, or are at risk for, autism/DD as well as the needs of their families or caregivers while improving their **quality of care**. Clinical and non-clinical trainees will understand their role in and **best practices** for **screening, assessment, diagnosis and providing evidence-based interventions** for individuals with autism/DD. Trainees will have the opportunity to observe autism diagnostic assessments at community clinics, engage in the training program "STAT-MD" (Screening Tool for Autism in Toddlers and Young Children, a training in conducting autism-focused assessment), participate in structured ADOS-2 (Autism Diagnostic Observation Schedule, 2nd Edition) training, and discuss autism red flags during feeding clinic assessments. Trainees will also learn communication skills and how to implement plain language to discuss their evaluation reports and recommendations for the family and caregiver. Lived experience mentors will provide on-going feedback to trainees.

Trainees will understand how **models of disability** (i.e., medical, social, cultural) and **lived disability experience** influence the expectations and decisions of individuals with disabilities through interactions with trainees and mentors with disabilities and those with lived disability

experience. Trainees will respect the perspective of individuals with disabilities and learn to situate their practices in **multidisciplinary service models** that are **family- and person-centered**, as well as **culturally and linguistically appropriate**. The WV LEND family mentor and mentor with a disability will be the leaders in these focused learning activities. Trainees with disabilities will have the opportunity to share their lived disability experience stories with others as a leadership training task. As part of the **Lived Experience curriculum**, trainees can participate in the following activities: MCH People with Lived Experience Learning Bundle, live sessions with people with lived experience, recorded talks, and podcasts. The WV LEND Family Mentor hosts hybrid weekly seminars discussing hot topics in disability and problems of practice through a lived experience perspective.

All trainees will participate as leaders in various **clinic and community placement opportunities** (e.g., feeding clinic, CED in-house clinics, community-based clinics, and family engagement activities) highlighting **emerging needs**, and **innovative methods** of healthcare for individuals with autism/DD through the lifespan. Trainees will understand the impact of a range of factors affecting the child as he/she **transitions from childhood through to adulthood** within community and healthcare systems by observing and engaging with individuals with disabilities and understanding the **lifespan developmental approach**. Trainees will understand the role **community health factors** play in health outcomes of individuals with autism/DD and how these factors influence the disability experience. By the end of the training year, through hands-on, mentored clinical interactions, clinical trainees will lead caregiver/client interviews, evaluations, and intervention sessions with children and their families/caregivers. During these interactions, **best practices for autism screening, diagnostics, and evidence-based interventions** will be at the forefront while also practicing **telehealth** delivery for assessment and intervention as needed to meet the needs of rural families. Non-clinical trainees will also participate and lead interdisciplinary chart reviews, family interviews and evaluation, and post-session meetings to discuss community supports and programmatic needs. All trainees will develop **quality improvement** skills by critically examining clinical practices and family engagement through group discussions and mentorship, using data, and measuring the impact of changes on outcomes and quality of life.

All trainees, clinical and non-clinical, will learn to critically appraise and synthesize scholarly **research** and discuss translation of the research to evidence-based practice and real life scenarios, including rural areas. Engaging with research in these ways enables trainees to evaluate new research findings particularly those related to autism/DD. Trainees will lead discussions during WVU and Rural Collaborative Journal Clubs, as well as utilizing the various concepts during clinical and community experiences, and group discussions. The Research Trainee Track and Badge Project both emphasize research skill development and application to **current and emerging disability issues**. The development and application of **critical thinking skills** for **quality improvement** in clinical, academic and community-based settings is a primary goal for all trainees.

The **MCH leadership framework** encourages trainees to reflect on their role as an individual as well as their role on an interdisciplinary team providing services and supports to children and youth with autism/DD, and those who care for them. All trainees will learn how larger **systems of care and service delivery models** impact not only children and youth with autism/DD and those who care for them but the trainee's own provision of service. Trainees will understand how **health policy** is developed and what their role is during this process. Trainees will understand systems issues by engaging with **Title V Programs,** and through review and discussion of **MCH population-focused legislation.** WV LEND encourages all trainees to follow a path of development of leadership skills through widening circles of influence beginning with a focus on self-developing skills through learning and experiences. As they near the end of their training trainees begin to influence the behaviors of others through their example utilizing effective, self-reflective, evidence-based practice. The ultimate goal is to create future leaders who will have a broader impact on the community through **policy** with responsive and respectful habits to influence change over time and high level **systems thinking**.

Conceptual training develops awareness, knowledge and critical thinking. Through conceptual training, trainees learn to examine and discuss issues, topics and concerns. Trainees acquire general and specific knowledge about autism/DD, incorporating knowledge of growth and development, health promotion, and maximizing optimal family-centered outcomes. Trainees also gain a broad public health perspective of differing social, cultural and health practices of various populations of the rural region. Trainees will become familiar with the special challenges facing practicing professionals who deliver services to children and youth with autism/DD across their lifespan and those who care for them in rural resource-limited settings. Conceptual training includes academic instruction but also focuses on the development and implementation of the association between concepts and critical thinking. Through conceptual training, trainees will understand the how and why of evidence-informed practice and disability service improvement utilizing inquiry-based learning and enhancing problem-solving skills. Trainees, over the course of the training period, develop an **e-portfolio** demonstrating their skills related to leadership, interdisciplinary training, academic and experiential participation. WV LEND faculty introduces these conceptual training experiences during **WV LEND Expo Orientation**.

Performance training assists trainees in developing customized skills and behaviors necessary for their future professional and personal commitments. Trainees will practice skills used in real life situations through microlearning sessions, leadership seminars, clinical and community experiential activities, family engagement activities, and community support education fairs. Instruction will also be provided through discussion, modeling, and mentor/peer feedback. By the end of the WV LEND Program, trainees will lead sessions and discussions utilizing these specific skills and cognitive strategies with interdisciplinary team members, families, children, and community partners.

Requisite skills are taught through a combination of conceptual and performance training activities. In care coordination skills training, for example, trainees add components of model practice in effective care coordination to their professional "tool box" through conceptual training. In performance training, trainees role-play, collaborate with mentors in care coordination and have primary responsibility for care coordination for child and family. Trainees discuss with mentors the needs of the populations in the care coordination systems for which they have performed. Thus, trainees gain leadership attributes that help them become catalysts for exemplary care coordination in both direct service delivery and systems enhancement. This is also a vital component for non-clinical trainee skills development. Then, lived experience trainees gain awareness and communication skills to support people with disabilities in the community, specifically in schools and healthcare settings.

The effectiveness of the Curriculum and Education content will be reviewed biannually by the Project Director, co-Project Director, Training Director, Curriculum Director and the Core Faculty. We will also seek input from current and former LEND trainees for continuous improvement of programming.

Academic

Academic content will be suitable for all trainees' skill levels and backgrounds by offering the information, such as group discussions and activities, synchronously, in-person and virtually (Zoom), as well as recorded for asynchronous participation at the trainee's own pace. Other content provided in written format will also be available in audio format, such as books and journal articles. Captioning is also provided during all live and asynchronous content. WV LEND will work with the WVU Office of Student Accommodations as needed for specific trainee needs. All of the online learning modules, such as the Introductory Disability Institute, are self-paced and available for completion at any time. Academic content is housed on the learning platform, SOLE, a WVU learning management system, which all trainees will have access to from the start of their training year. Trainee hour accumulation is monitored through monthly timesheets submitted from trainees, attendance logs and SOLE gradebook. Academic activities available for completion by all trainees include the Introductory Disability Institute, Disability Identity Badge**,** Trainee Tracks, Microlearning Sessions, Reading for Reflection and Growth, Independent Learning Modules, Following Bill, and Disability Studies course completion or audit.

To support the mission of improving the health of infants, children and adolescents who have, or are at risk for, autism/DD, **the content of the curriculum must contain a strong academic foundation**. Curriculum must provide training on appropriate state-of-the-art tools for screening, assessment and diagnosis of autism/DD, give trainees an understanding of disability theory, recognize the role of community health factors on health outcomes and present issues relevant to healthcare and transitions for young adults. Academic training content uses a variety of methods, including **independent readings**, **learning modules, trainee track learning activities,** and

distance learning to provide synchronous and asynchronous learning opportunities. Synchronous learning may occur both in person with a virtual option or only virtually (Zoom). Trainee outcomes are evaluated through self, peer and mentor evaluation.

AUCD National NDD curriculum resources are infused in academic training and provide for the basis of a flipped classroom in which trainees participate in pre-learning prior to seminars, clinics, and other discussion groups. **Independent readings** are recommended to be completed prior to LEND clinics and leadership activities so trainees come to the discussion with increased understanding of the topic at hand. Additionally, WV LEND has developed learning **modules/tracks** that provide for individualized, guided learning on a variety of leadership and disability topics. Through a series of modules, trainee tracks provide for more indepth learning in areas of trainee interest and are developed for advanced disability leadership, interprofessional collaboration, research, clinical with emphasis on rural care, autism/DD diagnosis and supports, MCH-focused legislation and improving systems of care. Trainees are required to select one major track, for 40 training hours, and at least one minor track, for 20 training hours.

Microlearning sessions will occur weekly for quick bursts (~20 minutes) of academic content to add to other weekly conceptual topics to enhance trainee understanding. For example, during a feeding clinic a patient may present with a feeding tube. The nutritionist/dietitian would then offer a microlearning session about different feeding tubes and blenderized food options. Microsessions will be led by WV LEND mentors in the fall semester. In the spring, trainees can be presenters of topics they have advanced knowledge or experience. We will have a microlearning session through the academic year entitled ”Johnny and Mary grow up - utilizing a life course perspective”. It is based on the medical, social and cultural models of disability .Some of the topics to be covered include early signs and how relates to screening, the recent information of gender differences in males and females with autism and the changing ratio of males to females identified with autism, school supports, community supports, family supports, transition to adult, voices of individuals with autism and the neurodiversity movement.

WV LEND provides instruction on the screening, evaluation, and diagnosis of children with autism/DD. Mastery of typical development is imperative in identifying children who should be referred for further screening and evaluation. The Training Director developed training videos with resources demonstrating developmental tasks for children. Developmental training videos can be used by either long-term and medium-term trainees to demonstrate typical development. The AUCD NDD National Curriculum provides resources about autism/DD that are incorporated into the LEND academic curriculum. Trainees learn about screening tools and their functions. Trainees, particularly in the interdisciplinary setting, learn their potential role in the diagnosis of children with autism/DD. Lived experience trainees will impart information on having difficult discussions and giving sensitive information to families.

To increase the number of practicing professionals who are knowledgeable about the healthcare and life needs of children and youth with autism/DD in rural areas and those who care for them, WV LEND is committed to providing educational opportunities to all WVU School of Medicine and WVU students from any department, community providers, students from other institutions, as well as other lived experience individuals. During their child development rotation, WVU Department of Pediatrics residents may participate in WV LEND activities with trainees, particularly the Feeding and Swallowing Clinic and Leadership Seminars.

The didactic content is further described in Attachment 5, Curriculum Forums.

Clinical

Interdisciplinary clinical and practicum experiences are a hallmark of WV LEND curriculum. During the exit interview, trainees cite the interdisciplinary clinics as one of the most important activities within the program. The clinic orientation for the Feeding Clinic and the LEND Support Program begins with a discussion by the clinic mentors as to their discipline's role during the clinic and as well as how they interact with other disciplines. Interdisciplinary care is a pillar to these clinical activities highlighting the value of including the patient and family as part of the team. Clinic etiquette for in person and telehealth visits are reviewed. In these clinics, trainees observe, evaluate, and implement interventions with children from infancy to eighteen with a variety of diagnoses. These children primarily come from rural areas of the state where local services are often limited if available at all.

These interdisciplinary clinics model a supportive environment that values and utilizes the skills and expertise of the team to arrive at outcome-driven and family-centered joint recommendations. Mentors demonstrate mutual respect and shared leadership across disciplines. The participation of the lived experience mentors and trainees in clinic activities ensure there is open dialogue between families and clinic staff which is patient/family-centered with plain language. Families are also invited to include members of the child's therapy and medical teams.

Prior to the visit, the mentor and trainee team meet to discuss the concerns of the family considering cultural, medical, behavioral, sensory, and development factors. Then, trainees directly (in the clinic room) or indirectly (by video) participate in clinic based on their discipline. During post-staffing all factors contributing to feeding challenges are discussed. A plan is developed along with the family. Parents are offered a digital recording of the session as a means of recalling techniques and information discussed during the clinic visit and receive written recommendations to take home. The feeding team also contacts the child's providers with recommendations and technical assistance. This will often occur as a three- or four-way visit between the feeding team, family and therapists/teachers. Telehealth sessions are used as needed by the family. Telehealth allows clinical activities to occur in the natural environment.

These clinics allow both clinical and non-clinical trainees to directly participate by gathering, synthesizing and developing interdisciplinary recommendations for the family. As time goes on the trainees take on greater leadership roles in these clinics. This model demonstrates the **application of interdisciplinary practice** in providing interdisciplinary care that is individualized, comprehensive, family-centered, culturally competent and coordinated as it addresses short, medium and long-term goals. Finally, trainees have the opportunity to discuss the interdisciplinary process among themselves and with their discipline mentors in the context of their work in the CED clinics. This discussion focuses on individual discipline concerns as well as how these concerns might overlap from one discipline to the next. These two experiential activities provide trainees with the opportunity to learn the roles of other disciplines as well as being introduced to the interplay of team member roles through observation of mentors from a variety of disciplines.

LEND trainees are expected to attend and participate in other interdisciplinary clinics which occur off campus from the CED. Examples of interdisciplinary clinics include the Cerebral Palsy and Spasticity Clinic, Complex Care Clinic, WVU Speech, Language and Hearing Clinic, speciality focuses on augmentative communication and audiology, and the CRADLE (Pediatric neuromotor assessment and functional rehabilitation) Lab.

The LEND Program and the WVU Department of Pediatrics provides an additional full range of opportunities for the appropriate trainee to demonstrate a capacity to evaluate, diagnose or rule out, develop and provide evidenced-based interventions to children and youth with autism/ DD. The WVU Medicine Golisano Children's Neurodevelopmental Center (NDC) provides diagnostic and ongoing care for children and youth with autism/DD based on a comprehensive, exemplary, interdisciplinary clinical services model which is family-centered and culturally appropriate. These diagnostic services are led by a LEND mentor. The NDC houses pediatric therapy services including physical therapy, speech therapy, occupational therapy, and ABA.

The majority of the children that attend these clinics are from rural and resource limited areas from around the state. Trainees experience the problems with access to care due to limited resources in rural counties and schools. They begin to understand the impact of the culture of poverty and Appalachian fatalism on the child with autism/DD and their family.

Trainees have opportunities to participate in programs within the CED including the Traumatic Brain Injury program, WV Assistive Technology System (WVATS), IMPACT (Rural Health Integration Models for Parents and Children to Thrive), and the Country Roads IPSE program. A program supported by the Paths for Parents (P4P) program is Project DOCC (Delivery of Complex Care). Project DOCC is a simulation for pediatric and family medicine residents, and nursing and pharmacy students to understand and experience how to impact delivery of care in a family/person centered way. P4P helps to empower parents to participate in the **medical home** as well as help families navigate medical, educational and community systems of care. The trainees have been able to participate in Project DOCC and other Paths for Parents activities.

Community

The interdisciplinary training, practice and application model helps trainees understand how to develop **public health impact programs of care for those with disabilities.** Modules are available through the MCH-focused legislation and improving systems of care trainee track. During leadership seminars and the research trainee track, students will become familiar with data sets such as the Child Health Data comparing WV children in CSHCN and not in CSHCN with national data. This provides perspective as to comparison of the health and well-being of WV children with and without disabilities with national and regional statistics. Disability impacts the health of an individual and the family but that individual is affected even more so by the other health issues common to the community or region. WV has a disability prevalence rate of 19.5% compared to the US population of 13.4%. The rate of obesity in children with disabilities is 41% higher than for children without disabilities. As the obesity rate for WV is one of the highest in the country it would follow that the obesity rate in WV children with disabilities would be even higher than in those without disabilities. The National Survey of Children's Health allows for the comparison of health and well-being of children living in rural areas versus those living in urban areas. Factors such as this make it important for trainees to have a public health perspective of care and to engage in activities that promote solutions through interdisciplinary models.

Individuals with autism/DD do not live in isolation and are part of a family and larger community that can provide supports as these individuals progress through the lifespan. Through the LEND Support Program, trainees have an opportunity to support families in their journey through life with their children and youth. Individual Action Plans will be developed through shared decision by trainees, mentors and families. In order to be able to provide this support, trainees need to learn about a number of topics including educational services and IEPs, healthcare challenges of the particular disability and transition resources. Trainees are to engage in self learning, as well as leadership seminar activities, in order to provide appropriate information to children and youth and their families.

The impact of **systems of care** for those individuals with autism/DD and their families is approached from a didactic level to a practical level all the while encouraging use of an interdisciplinary lens. Through the LEND course work trainees gain insight into systems of care through the individual disability level, levels of systems and ultimately through policy. As part of the **Introductory Disability Institute** and the **Medium-term trainee Disability Immersion**, long-term and medium-term trainees will complete modules on disability theory, models of disability, and neurodevelopment and autism. They will reflect how these concepts guide the view of disabilities and systems of care. Throughout LEND activities these concepts will be brought into the conversation. Also, within the **Introductory Disability Institute,** readings and activities provide trainees with the fundamentals of systems of care. Through the **Family Engagement Activities** and clinics, they see the impact of approaches to systems of care on the individual with disability and their family.

Volunteer community-based activities allow a different perspective of systems. Trainees participate in the NDC clinics and LEND Support Program which provide trainees with the opportunity to directly see the effects of integration of services. As they are seeing children and families from around the state they not only gain a better understanding of the disparity of services in a rural resource limited state but also work to problem solve to help families. For example, adverse childhood experiences occur more frequently to children living in rural areas than those living in urban areas. Trainees will be asked to review these differences of services in the more rural areas of the state. Pre-staffing for LEND clinics will include a discussion of the demographics of the county, school system, major employers and other factors that affect an individual with a disability. This will be another opportunity to reflect upon the community health factors that influence disability services and care.

Leadership

MCH Leadership competencies are infused throughout all curriculum forums and promote a developmental progression of increasing spheres of influence. The MCH leadership competencies are the framework that provides the scaffolding for all academic and experiential activities. This is reflected in Attachment 5: Leadership Competences by Curriculum Forum.

During the **LEND Expo Orientation**, trainees first begin to learn about the MCH Leadership Competencies and other foundational concepts, as well as WV LEND policies and procedures. As part of the orientation, trainees meet with their assigned mentors, discuss their goals and develop their **Individualized Training Plan (ITP)**. Each trainee, with support from their mentors, will focus on at least one leadership goal, one research or policy-focused goal, and one personal goal during their LEND training. Trainees work with their mentors to develop a stepwise plan to reach these goals, as well as discuss supports needed for successful completion.

The **Introductory Disability Institute** activities occur through on-line modules and learning activities. An initial group meeting is held to introduce institute activities. Trainees will review and reflect on the AUCD trainee orientation modules. Each trainee will complete the MCH Leadership assessment as an initial **self-reflection**. The assessment results will be shared with the training director and mentors and will form the basis of the Individualized Training Plan. The learning activities will cover topics introducing **knowledge of MCH base and context,** as well as understanding the history and structure of MCH. Modules will introduce MCH concepts of family-centered care, medical home, Life Course Health Development Model, and models of disability. Trainees will complete the modules on neurodevelopment, autism, education legislation, introduction to research and evidence-based practice and interprofessional collaboration.

This is followed by cooperative learning experience incorporating the fundamentals of leadership based on the MCH Leadership Competencies. The Leadership Seminar introduces **ethics**, **communication**, **negotiation and conflict resolution, community health factors, coaching**

and mentoring, systems thinking and policy. Through the Journal Club trainees utilize **critical thinking** to critique research articles. The clinical experiences go beyond their individual discipline clinical experiences by emphasizing the importance of **family/professional partnerships** as well as **interdisciplinary/inter-professional team building.**

The model of self-reflective practice as professionals provides a basis for **developing others through teaching, coaching and mentoring.** It will serve as a building block for trainees in the development of their own reflective future learning and leadership.

Trainees have an opportunity to attend a variety of interdisciplinary clinics to see MCH competencies in action from different sites which include the Feeding and Swallowing Clinic, LEND Support Program, Neurodevelopmental Clinic, Complex Care Clinic, and other CED and community-based clinics and programs. Through these experiences, trainees gain knowledge about children and youth with autism/ DD as well as the impact of **systems of care**. Specifically, the LEND Support Program allows trainees to synthesize what they have learned about public policy, advocacy and programming to help families negotiate systems of care. LEND Support Program encourages evidence-based treatment, consultation with local providers and exploration of the service delivery models in a rural state with a variety of service gaps. **Self-reflection** is ongoing throughout the training and is captured in the trainees' **Reflective Practice Activities**. In the exit interview, trainees reflect upon experiences, the goals they have obtained and their leadership goals for the future. It comes full circle as during the spring LEND graduation, trainees reflect upon their experiences and future plans and how LEND has supported these.

Partnerships

WV LEND is housed within the WVUCED and **serves as WV LEND's partner organization.** It is a federally funded designated University Center for Excellence in Developmental Disabilities (UCEDD) and is part of the national network of 68 UCEDDs. This network is coordinated through the Association of University Centers on Disability (AUCD), a national association that supports members to promote quality of life for people with disabilities and their families. The mission of the WVUCED is "to improve the lives of West Virginians by providing evidence-based services and training opportunities, conducting research to continually pursue new and innovative approaches to care and support, and share information about supports and resources throughout the state and nation" (WVUCED, 2026).

Partnering with the WVUCED allows WV LEND trainees to access a wide range of programs, expertise, and interdisciplinary resources within the disability field. The WVUCED includes multiple state and university-based initiatives that support children and youth with autism/DD and their families across the lifespan. These include the Disability Studies academic division within the WVU School of Medicine, the WV Positive Behavior Support Program (PBS), the WV Assistive Technology System (WVATS), Specialized Family Care and the WVU Feeding and Swallowing Clinic. Together, these programs provide comprehensive services to support

children and youth with autism/DD across the lifespan. They also create training opportunities for WV LEND trainees in interdisciplinary clinical practice, community-based service delivery, assistive technology applications, academic supports, and leadership development.

Clinical partnerships are central to the mission of WV LEND. Collaborative clinical/medical partners include both WVU Medicine and WVU Medicine Golisano Children's Hospital. WV LEND provides training for medical residents and practicing providers. WVU Medicine and WVU Children's provide training like Grand Rounds and allow trainees to observe clinics. WVU NDC is also a primary clinical partner with WV LEND. The NDC supports participation as a WV LEND mentor for their developmental pediatrician. LEND does buy out partial FTE for this mentor. The NDC also allows trainees to observe developmental assessments, ABA clinics, and speech, occupational, and physical therapy services.

WV LEND maintains partnerships with WV Title V Programs to strengthen statewide systems of care for children and youth with autism/DD and their families across the lifespan. WV Children with Special Health Care Needs (CSHCN) Program provides a Leadership Seminar to trainees introducing WV's Title V infrastructure, available services, and strategies for coordinating care across systems. CSHCN also refers patients to the WVUCED Feeding and Swallowing Clinic. The LEND Nutrition mentor will be providing TA to the CSHCN program. WV LEND also supports at least one provider from WV Birth to Three (WVBTT), WV's Title V state early intervention program, as a trainee. In addition, WV LEND mentors provide coaching, training (CE), and collaboration with WVBTT providers who need additional support when providing services to children with autism/DD and their families. These partnerships promote workforce development, improve care coordination, and strengthen early identification and intervention for children and youth with autism/DD across WV's statewide system of care.

WV LEND, in partnership with the competing Blue Ridge/Virginia, Alaska, and Missouri LEND Programs, share a mission of improving the lives of children and youth with autism/DD and those who care for them across the lifespan in rural areas. WV LEND will continue membership in the LEND Rural Collaborative, a national forum that brings together LEND mentors and trainees to advance innovative, rural-focused training and systems change initiatives to increase the rural workforce equipped to serve children and youth with autism/DD and their families. The Rural Journal Club has been a successful joint activity. The members of the Collaborative take turns hosting the Journal Club. Mentors from each program work with their trainees to identify a rural pertinent article. The article is shared with the other LENDs and then the trainees lead the discussions which are very rich.. Trainees from other LENDs have participated.

The LEND Rural Collaborative serves as a forum for LEND leadership where faculty and trainees can share and develop rural-specific strategies related to: 1) increasing access to evidence-based direct clinical care for autism/DD; 2) leveraging information technology for direct care, training, and continuing education; 3) developing curriculum materials focused on working with rural populations; 4) increasing the workforce of providers with training in

autism/DD in rural areas; and 5) leadership across a wide range of community sectors. The LEND Rural Collaborative provides a unique opportunity for trainees to expand their knowledge of the strengths and the challenges of service provision and systems of care in a variety of rural areas and to collaborate with trainees and mentors from other rural LENDs. During the project period, the group has agreed to jointly provide at least two CE events during the course of the grant cycle and rotate responsibility for hosting Rural Journal Club.

To strengthen training in community systems of care, WV LEND will partner with the Disability Action Center (DAC). In collaboration with the DAC social worker, a former trainee, WV LEND will expand experiential, onsite training that allows trainees to directly engage with adults with autism/DD across the lifespan and their families. Trainees will gain firsthand experience in community outreach, interdisciplinary service delivery, and family-centered support through participation in programs including Career Readiness, Experience-IT Co-Op with Homestead Farm Center, Health and Wellness, and DAC Connect. WV LEND provides financial support to the DAC social worker as the program's Community Social Work Mentor.

Academic partnerships across WVU are foundational to WV LEND's interdisciplinary training model. Applicants are recruited from multiple departments and disciplines across WVU, with multiple academic programs recognizing WV LEND as a premier leadership training opportunity. Several programs, including the Divisions of Communication Sciences and Disorders, Physical Therapy, Occupational Therapy, Physician Assistant Studies, and College of Law actively promote WV LEND to their students and encourage qualified applicants to apply.

WV LEND provides financial support for a faculty mentor from the WVU SOM Division of Physical Therapy. This partnership enables trainees to attend the WVU CRADLE Lab, a pediatric neuromotor assessment and functional rehabilitation lab, where they receive hands-on, interdisciplinary training in infant motor development, developmental milestones, movement assessment, and evidence-based approaches to early identification and intervention for children with autism/DD. These experiences strengthen trainee competency in developmental screening, collaborative practice, and family-centered care.

Letters of support and agreements can be found in Attachment 3: Agreements with other entities.

Technical Assistance and Continuing Education

The provision of Continuing Education (CE) is provided by WV LEND mentors. Trainees could have roles in the delivery of CE with mentor supervision. There will be at least three continuing education activities per year throughout the duration of the project period. Mentors currently provide continuing education through grand rounds and presentations at conferences and workshops throughout the state, region and nation. WV LEND, through the WVUCED, is an approved continuing education provider for a variety of disciplines including social work, psychology, occupational therapy, speech therapy, physical therapy, and nursing.

In the next year, WV LEND has multiple CE opportunities planned for practicing professionals. First, WV LEND has partnered with Monongalia County Schools centered in Morgantown, WV to provide CE on complex feeding needs in students with autism/DD in the schools. An expansion of CE opportunities is also a technical assistance(TA)/coaching model developed with Monongalia County SLPs, OTs, aides, and nurses to support feeding for specific students. The model includes in-school support from WV LEND mentors in community schools to prioritize positive outcomes for students with autism/DD who have feeding challenges. WV LEND mentors will provide assessment and treatment planning alongside school therapists, teachers, support staff, the student, and the student's family. WV LEND will also connect students to community supports and programs that will promote optimal outcomes.

In addition, WV LEND provides TA/coaching for children and youth with autism/DD enrolled in CSHCN who need additional support with nutrition and feeding. WV LEND mentors provide interdisciplinary feeding assessments and then collaborate with CSHCN staff to develop a plan to share with the child and caregivers. Currently, CSHCN does not have a nutritionist/dietician or other therapy providers on staff so TA/coaching for CSHCN staff closes the gap for many children and youth with autism/DD who would not otherwise have access to a feeding team who focuses on complex feeding needs.

Similarly, WV LEND mentors provide TA/coaching to a statewide therapy company, LinguaCare, who also provides SLP services locally. LinguaCare has monthly CE sessions with mentors followed by mentor "office hours" to collaborate on complex clinical cases or challenging access needs for children and youth with autism/DD and their families. "Office hours" technical assistance ranges from clinical support to problem solving for service needs.

WV LEND also provides TA/coaching to other WVUCED programs. Assistance with topics such as language use, interdisciplinary practice and collaboration, and assessment/intervention for children with autism/DD and their families has been provided to PBS, WVATS, and IMPACT WV. WV LEND will continue to provide TA to WVUCED Programs as requested. In addition, WV LEND will be presenting at WVUCED Grand Rounds to staff, including practicing professionals, four times in the upcoming academic year.

The WVUCED Mountain State Conference on Disabilities scheduled for April 2027 will include a track with sessions presented by WV LEND mentors. Sessions will include evidence-based interdisciplinary clinical intervention for children and youth with autism/DD and their families across the lifespan; gender differences in autism symptoms resulting in females being identified later; community engagement for children and youth with disabilities; family-centered practices; screening, assessment, diagnosis and treatment for children and youth with autism; innovative practices for workforce expansion; and current leadership and research initiatives in the field of autism/DD.

Currently, multiple WV LEND mentors also have conference submissions for local and national conferences being reviewed by conference committees. Mentors commit to yearly presentations at conferences, guest lectures in Disability Studies courses, and participation in local community health fairs, service initiatives, and information booths and tables at community events.

Each year a communication plan is developed in consultation with the Dissemination Officer. It is an opportunity to review the promotion of LEND activities including continuing education. Fliers are developed to advertise available trainings. Available WV LEND trainings are advertised through WVUCED, WVU and AUCD websites and email. WV LEND trainees can attend these trainings to earn LEND training hours. Attendance by trainees provides an opportunity to network and collaborate with other professionals with similar interests.

The LEND Rural Collaborative has agreed to develop two continuing education opportunities over the course of the grant. The CE opportunities provided by the LEND Rural Collaborative are to be determined once the next academic year begins.

TA has been provided by and will continue to be provided by WV LEND mentors. Mentors provide technical assistance on the state level by serving on numerous advisory boards including the WV Children with Special Healthcare Needs Medical Advisory Board, the WV Infant and Toddler Mental Health Association Board, and Governor's Early Childhood Advisory Council. These are considered important activities by the mentors and the departments that support them. These activities will continue through the upcoming grant cycle. Mentors will continue to provide technical assistance through membership on boards, leadership positions in discipline organizations, leadership positions in disability organizations and assisting in policy development. Trainees are encouraged to participate in activities as they are able. Technical assistance available will be advertised through the WVUCED website.

High-Level Work Plan

The Work Plan is provided in Attachment 1.The chart provides the goals and objectives to be met with each LEND cohort, the activities to obtain those objectives, who will be responsible for each objective and the measurement to be used. The timeline is presented in relation to each LEND cohort as it was felt that this would provide a clearer picture of

The Trainee Leadership Competency Chart (Attachment 5) identifies trainee MCH Leadership competencies and the curriculum forum that addresses each competency. The Syllabi for Curriculum Forums (Attachment 5) identifies by activity the purpose, method of learning, content, time commitment and method of evaluation for each curriculum forum.

Resolving Challenges

WV LEND incorporates SWOT (Strengths, Weaknesses, Opportunities, and Threats) analysis as part of its continuous quality improvement (CQI) process as a means of identifying emerging

challenges/threats/weaknesses, developing opportunities to capitalize on strengths, and develop data-driven strategies for problem solving. Specifically, when potential challenges are identified, WV LEND applies a structured SWOT analysis, developed by mentors, faculty, and community partners as appropriate, to examine contributing factors and identify potential barriers to successful challenge resolution. A collaborative approach is utilized to develop targeted action plans, assign responsibilities, establish timelines, and monitor progress. Implementation of the SWOT systematic approach promotes timely problem-solving, accountability, and sustainable CQI. Even more, utilization of SWOT analysis is a skill that builds over time so challenge resolution becomes more succinct over time.

For example, communication with current and past trainees has historically been a weakness within WV LEND. Strong team dynamics, engaged trainees and targeted communications, trainees were not responding to communication attempts via email, telephone, social media, nor traditional mail. WV LEND Core Faculty met numerous times to problem-solve increasing trainee response to communication in a more timely manner and with more detailed responses. SWOT Analysis was conducted to develop an action plan, assign tasks, develop a timeline and monitor progress. The team identified generational learning and hybrid approaches to learning as opportunities and flexibility and team experience as strengths. Based on these factors, the idea of incorporating secure, web-based texting as a communication platform emerged. The project administrator was charged with initiating use, the team developed a timeline of evaluation, and progress of the new procedure was monitored. Web-based texting improved trainee communication and moved to an opportunity for WV LEND to expand. Other "threats" and "weaknesses" WV LEND is likely to incur over the training period are listed below. SWOT Analysis will continue to be implemented as an approach to resolve these challenges.

TABLE 1: WV LEND SWOT ANALYSIS

Strengths	**Weaknesses**
● Interdisciplinary team experience and expertise ● Access to HSC and WVU ● WVUCED connection ● Extensive trainee pool ● Rural state access for community connections ● Lived disability experience multiple of mentors and trainees ● Flexibility	● Location of WVUCED building ● Isolation from day-to-day collaboration opportunities (WVU campus/WVU Medicine) ● No special education department at WVU ● Communication with current/past trainees

Opportunities	Threats
● Trainees from other institutions to access other disciplines ● Create hybrid/ asynchronous approaches to learning ● Generational learning concepts applied to trainings ● Web-based texting ● Recruit trainees from other in-house programs	● Social media misinformation ● Financial threats ● Loss of training programs ● Trainees with disabilities may lose benefits if accept LEND funding

Performance Reporting and Evaluation

Performance Measurement and Reporting

Program performance measurement data is collected using multiple quantitative and qualitative methods, including MCH trainee competency self-assessments, mentor evaluations utilizing rubrics, training participation records, clinic and Leadership Seminar activity logs, ITPs, training evaluations, LEND follow-up surveys, exit interviews and HRSA-required measures.

Performance Data is reviewed by PD, co-PD, TD, PM, and WVUCED Data Analyst during monthly data meetings to inform program improvements and ensure high implementation fidelity. PM and TD will collect, organize and prepare trainee data for monthly meetings. Program Administrator enters mentor data (i.e., CEs offered, TA, products) into CEDWVU database (CODA) and TD enters trainee data (i.e., training evaluations, hours) into WV LEND Online Learning Platform (SOLE). WVUCED Data Analyst will collect, organize and prepare initial data analysis prior to meetings. Data Analysts may identify CQI data and create run charts to assist in formative programming decisions.

TABLE 2: WV LEND Performance Evaluation Methods

Measurement Objective	Evaluation Method(s)	LEND Goal
Trainee Recruitment	Number and discipline of long, medium, and short-term trainees	1
Mentor Performance	Mentor evaluation rubrics	1
Trainee Retention	Number of trainees who complete program, qualitative feedback from exit interview/follow-up surveys	1, 4

CE Offerings to Practicing Professionals and the Community	Training evaluation rubrics, number/discipline of learners and CE occurrences	1, 4
TA	Number of occurrences and participants, training evaluation rubrics	2, 3
Annual Diagnostic Evaluations	Number of autism/DD evaluations completed	2, 3
Activity Impact	Discussion Rubric, training evaluation rubrics	1, 2, 3, 4
Interdisciplinary Trainings	Number of participants and occurrences	1, 2, 3, 4
Trainee Performance	Trainee evaluation rubrics, mentor feedback, ITP, knowledge assessment, records of attendance, ISR, peer/self/mentor presentation evaluations, documented skill acquisition, pre/post tests, assessment of Individualized Action Plans (IAP)/parent satisfaction, products, badge meetings, Journal Club participation, Lived Experience Mentor Evaluation	1, 2, 3, 4
Rural Collaborative	Number of meetings, activities, products	3, 4
Product Dissemination	Number of products and products shared, size of audience	1, 2, 3

All proposed outcomes of the LEND Program are entered into established secure (two-step verification) online systems. These systems include an internal WVUCED tracking system (CODA) and the AUCD web-based NIRS system. Both systems have friendly interfaces and reports can be customized and retrieved by the LEND team. NIRS enables network members to manage data on their training programs, projects, activities and products, to support compliance of federal reporting requirements. The WVUCED Data Analyst is responsible for establishing accounts for all WV LEND mentors and for assisting with data entry and reporting activities. This includes details on TA and CE, including type, number of participants, evaluation feedback, dissemination, training/service activity by recipient with demographic information where indicated. Assistance of WVU Health Science or SOM statisticians may be used for organizing and analyzing data. The WV CSHCN Program will assist in the review and presentation of data obtained in relation to collaborative projects.

Specific to 5-year post-training tracking and reporting, WV LEND will implement multimodal contact attempts, including web-based texting used during the trainee's time with WV LEND, email, phone calls, on-going social media contact and media announcements via LEND/WVUCED webpages and social media accounts. On-going reminders of the importance of maintaining trainee contact post-program completion will also be provided throughout the duration of the training period. WVU LEND is planning a CQI product to determine the most effective method(s) of communicating with past trainees.

Program Evaluation

WV LEND employs a comprehensive performance measurement and evaluation framework to monitor adherence to program design, assess trainee outcomes and practicing professional knowledge acquisition, evaluate overall program impact and inform Continuous Quality Improvement (CQI). On-going evaluation ensures program goals, outcomes, and objectives match LEND performance measures while still adhering to MCH guidelines and principles.

This evaluation strategy is designed to:

- Monitor implementation fidelity to ensure WV LEND is moving at the pace and in the direction originally planned and each activity is conducted and evaluated as described in the approved Work Plan.
- Measure outcomes of WV LEND to determine the impact of program activities in increasing the capacity of the workforce skilled in provision of services to individuals with autism/DD and developing future leaders in the screening, identification, evaluation, diagnosis, and intervention of autism/DD.
- Provide timely data during monthly data meetings to support ongoing CQI so LEND Core Faculty can make modifications in activities if predicted outcomes do not occur and also to measure and capture program successes. Data sources could include, trainee surveys, application information, required DIGS measures, CE training surveys, attendance logs, self/peer/mentor evaluations, self-reflection and content competencies.
- Collect and organize data to assist HRSA in evaluation of LEND programs, including DGIS measures and Autism CARES Act reporting standards.
- Provide direction for the sustainability of the program activities so the most effective and high impact initiatives and partnerships can continue and be expanded.
- Develop and disseminate shareable results and outcomes to inform a larger audience regarding WV LEND impact (audience- other LENDs, community, WV Congressional individuals and related public initiatives).
- Inform federal reports to Congress by participating in the evaluation of the Autism CARES Act Awardees.

WV LEND anticipates and prepares for evaluation barriers. Some predicted barriers and plan to mitigate include: missing or partially completed trainee surveys, follow-up with trainees

individuals and use automated reminders; measuring leadership change/growth over time, use multiple-measures/tools such as self/peer/mentor evaluations and reflection for assessment; and changes in reporting requirements, monitor HRSA updates and attend national evaluation meetings.

PD, co-PD, TD, PM along with WVUCED Data Analyst, meet monthly to discuss data outcomes and design changes to implement within the current training year. Changes can be implemented and measured for improvement within the same trainee cohort. Core Faculty include multiple WVU faculty members with extensive experience in formative program evaluation experience, quality improvement and leadership. As a result, data-driven change can be identified in a timely manner, designed and implemented in a short period of time within the training year to optimize trainee experiences. The WVUCED Data Analyst attends national UCEDD and LEND data meetings ensuring the utilization of the most current best practices, adherence to HRSA reporting guidelines, and maximized impact of collected data to promote program outcomes.

Continuous Quality Improvement (CQI)

WV LEND utilizes a CQI framework based on the **Plan-Do-Study-Act (PDSA) Model.** Evaluative findings are based on questions like *what are we doing?* and *how can we do better?,* then implementing changes. Iterative cycles are implemented, reviewed, assessed, with changes made and then monitored over time. Then, the cycle repeats resulting in formative positive change in an evolving and responsive training program. Performance outcomes are compared to established benchmarks and yearly objective measures.

Figure 2, below and Table 1 page 24, describes how process and outcome information can lead to modifications when necessary. This table shows how evaluation results are available to the funding agency. To determine if LEND has led to the overall goal of improving the health infrastructure and systems of care for individuals with autism/DD and their families, a series outcome evaluation method will be implemented. This utilizes a continuous improvement approach, the Model for Improvement: **PDSA**. The needs of individuals with autism/DD and their families are utilized to **PLAN** for the development of the system level goals and objectives. The competencies necessary for trainees to achieve, as part of their MCH LEND training, have been identified. These competencies provide a foundation for goal achievement and objectives (**Plan**) of the LEND Program. A competency-based academic and experiential curriculum (**Do**) has been developed, based on and modified by the outcomes of the current WV LEND Program. In the (**Study**) phase, process information is obtained through rigorous data collection utilizing established WVUCED data collection processes. All trainees and mentors submit data to the Data Coordinator who is responsible for overseeing data collection and entry into the WVUCED established online data system. The Data Coordinator also provides reports tabulating in-process information throughout the award period. Extracted data is then uploaded to the NIRS database. The impact of the program (**Act**) is measured in outcome data through such sources as the required performance measures and state performance on such measures as Healthy People

Goals. Through evaluative and CQI data, on an at least yearly basis, WV LEND curriculum and activities can be modified to meet the needs of an ever-changing and innovative healthcare environment. The Program Director, Co-I, Project Manager, and Training Director work together using a learning collaborative to ensure that CQI occurs. Trainees participate in an exit interview and provide feedback on their LEND experience as part of WV LEND's CQI.

Trainee leadership development serves as its own PDSA cycle for all trainees including, long, medium and short-term trainees. Trainees identify their leadership qualities and, with the Training Director and input from their mentor(s), develop their Individualized Training Plan (ITP). Review of the MCH Individual Self-Reflection (ISR) with trainees on a monthly basis by mentor(s) and the Training Director helps guide and monitor trainee progress. Halfway through the training year, the Training Director evaluates overall performance through activity completion, overall level of engagement, and mentor evaluation. Areas of strength(s) and need(s) for improvement are identified and adjustments in the curriculum and trainee participation are made. At the end of the training year, the Training Director, mentor(s) and each individual trainee review their ITP and ISR, and then reflect on the trainee's growth and plans for the future. Rubrics are developed to assist in standardized evaluation of trainees by self, peers, and mentors. Observational evaluation of trainees will occur through the use of pre-designed rubrics. For example, a Discussion Rubric is utilized to evaluate the effectiveness of trainee communication and engagement during active discussions.

CE and TA also create additional PDSA cycles. Training evaluations, on-going monitoring of attendance counts at CE events, qualitative feedback from CE/TA activities and adherence to national licensure standards create a framework for the PDSA Model questions of *what are we doing?* and *how can we do better?* These questions, paired with CE/TA evaluation data support the iterative PDSA Model cycles.

CQI findings are used to strengthen trainee experiences, enhance interdisciplinary education, identify gaps in trainee support, and ensure WV LEND curriculum and programming remains responsive and innovative to the emerging and complex needs of individuals with autism/DD and their families- especially in rural communities. CQI reporting occurs at least twice yearly to mentors, trainees, advisory committees, and other leadership to promote accountability, transparency, adherence to HRSA priorities, and sustained excellence within WV LEND in training leaders equipped to improve systems of care for children and youth with autism/DD and their families across the lifespan. Evaluation data can be organized then analyzed by local (community), state, regional, and national level activities then compared by location to ensure effectiveness across all training levels/locations. Project results and data outcomes are disseminated to multiple stakeholders, learners and community partners including, the CED Advisory Board, other LENDs and WV legislators and other government officials. Project results are shared with support from the WVUCED Dissemination Officer, through annual fact sheets and during LEND/AUCD meetings.

Sustainability

WV LEND is housed within the WVUCED. The institutional commitment provided to WV LEND from the WVUCED has been in place for over 20 years. LEND and CED share supports, resources, faculty and leadership. The established infrastructure supporting both entities includes dissemination efforts, a focus on workforce development and leadership development.

The impact of WV LEND is recognized and valued by existing partners. The WVU College of Law, WVU Department of Pediatrics and WVU Physician Assistant Studies Program all provided letters of support outlining their continued support to WV LEND and each remarked how LEND training enhanced the departmental education of their students. The WVU Department of Nutrition and Foods has expanded their curriculum so all graduate students complete a week long dietetic internship with WV LEND to build student capacity to serve children and youth with autism/DD and their families from a family-centered, strength-based approach. Together with academic partners, WV LEND is developing a leadership pipeline across multiple disciplines. Interdisciplinary collaboration during clinics and chart reviews, hybrid approaches to instruction, Rural Journal Club and responsive implementation of date-driven project change, have all proven to be effective strategies in improving LEND programming and in strengthening longstanding partnerships between LEND and the NDC, WV CSHCN, and WV Birth to Three. Recent partnerships are built on WV LEND's strong mentor presence including leaders in evidence-based practice in autism/DD, mentors with lived disability experience, and mentors embedded in WV's community and state service organizations.

Looking forward to future sources of financial support, WV LEND mentors actively seek additional and complementary funding opportunities that expand and help sustain interdisciplinary leadership training and continued increase in workforce capacity to provide exemplary support to children and youth with autism/DD and their families across the lifespan. By committing to CQI and implementation of data-driven change, WV LEND is well positioned to seek additional funding. WV LEND currently engages in cost-sharing with WVUCED in office space/rent/technology and faculty/staff salaries for resources such as accounting, communications and data management.

WV LEND recognizes sustainability challenges will likely emerge. For example, recruiting and maintaining mentors and providers in WV who are experts and leaders in autism/DD is historically a challenge. WV LEND proactively offers flexibility for mentors, engages alumni to develop future mentors and integrates LEND as much as possible into existing academic and clinical experiences. Currently, WV LEND has three mentors who are graduates of the program. Training and recruiting trainees to continue to provide exemplary services to children and youth with autism/DD and their families across the lifespan in rural Appalachia creates a strong positive impact on overall public health.

WV LEND is currently experiencing a transition in leadership due to the retirement of the previous PD. The previous PD is transitioning to a support role as co-PD. The current PD has been mentored by the previous PD when they were in the role of Project Manager.

Dissemination of project goals and outcomes to community agencies, stakeholders within the disability community, academic units, practicing professionals and national audiences will support continued improvement in the system of care for children and youth with autism/DD.

Organizational Information

This section will describe the administrative and organization structure within which the program will function. WV LEND is housed within the WVUCED which is under the WVU School of Medicine. The WVUCED and WV LEND have the mission to improve the lives of West Virginians with disabilities through evidence-based practice, training, information dissemination and the pursuit of innovative approaches to care. WVUCED also houses the WVU Department of Disability Studies within the WVU School of Medicine. Multiple WV LEND Core Faculty have appointments within Disability Studies including, PD, TD, and Disability Studies mentor. See Attachment 4 which provides an organizational chart which denotes the reporting relationship between WVUCED and WV LEND. Attachment 2, Staffing plan and job description, outlines the staff positions which will support the execution of WV LEND including roles. Further detail is provided in the section describing the Project Director, LEND faculty and staff qualifications found on page 37. Biosketches are provided for key personnel.

WV LEND has had a successful training program for over 25 years and has had long standing relationships with programs within WVU and WVU Medicine (See Attachment 3 Agreements with other entities - Departments of Pediatrics, Divisions of Physical Therapy, of Communication Sciences and Disorders, of Physician Assistant Studies and College of Law). Further support for trainee training has come from State Title V CYSHCN and the LEND Rural Collaborative. WV LEND has demonstrated an ability to engage trainees. Review of data (Funding tier calculations) from this and the previous grant cycle indicates that WV LEND has averaged 19 trainees per year. For the upcoming year, if funded, trainees will be chosen from over a hundred applicants from more than 9 disciplines. WVUCED funds a training director for those trainees who are associated with other WVUCED programs. The WVUCED and the WV LEND training directors work hand in hand.

The primary setting for LEND activity occurs at the CED Building, Research and Office Park, strategically located between the WVU downtown campus and WVU Robert C. Byrd Health Sciences Center in Morgantown, WV. Campuses are linked by the Personal Rapid Transit (PRT), a monorail system that allows students and faculty access to any of three campus locations within ten minutes. LEND will use 2,170 square feet of the CED's 24,420 square foot facility, which is accessible according to the Americans with Disabilities Act (ADA) standards. Resources available include individual staff and mentor office space; classroom space with

telecommunication support; dual clinic space with observation areas (used for Feeding and Swallowing Clinic and for LEND Support); and small (6-8 individuals) and large (60-70 individuals) meeting spaces. A trainee suite with 18 individual carols and a resource room allow for both formal and informal interaction among trainees, faculty and other professionals. LEND will incorporate existing supports, capabilities and resources to complement the development of family-centered interdisciplinary training specifically designed to address the needs of children with autism/DD and their families. The WVU Medicine Golisano Children's Neurodevelopmental Center provides a site for autism/DD evaluation and intervention allowing for the involvement of trainees. Additional clinical sites are part of WVU Medicine Golisano Children's Network.an array of exemplary, comprehensive, community-based services in a variety of institutional and rural community-based settings. Telehealth clinics allow for access to patients. In addition to these settings, trainees and faculty participate in a variety of activities in diverse community settings around the state and region. Early Intervention (WV Birth to Three) sites throughout the state are available resources for LEND trainees.

Accommodations are provided for trainees in conjunction with the WVU Office of Accessibility. See Academic section page 18. WV LEND has successfully met the needs of multiple trainees with specialized educational needs including hearing, vision and mobility.

WV LEND has the organizational capacity to gather, manage and use data to inform processes and advance outcomes.

Project Director, LEND Faculty, and Staff Qualifications

Project Director

The Principal Investigator/Project Director (FTE 65%), Cassaundra Corbett Miller, EdD, MS, CCC/SLP is a board-certified and licensed **speech language pathologist**. Dr. Miller holds a Doctor of Education (EdD) degree and a Master of Science in Speech Pathology with Disability Studies Certification. She has 20 years experience providing evidence-based communication and dysphagia evaluation and intervention for children and youth with autism/DD. Dr. Miller is ADOS-2 (Autism Diagnostic Observation Schedule, Second Edition) certified. For over a decade she worked as a pediatric speech language pathologist for the WV Birth to Three program with an emphasis on dysphagia evaluation and intervention for young children with autism/DD, including facilitating caregiver participation in intervention tasks, promoting caregiver education and connecting families to community resources. Dr. Miller is the Director of the WVUCED Feeding and Swallowing Clinic. She provides clinical supervision and mentorship for speech pathology graduate students and is the **Speech Language Pathology Mentor** for WV LEND. Her areas of research interest are implementation of coaching models for practicing providers and family-centered feeding therapy practices for children with autism/DD. Dr. Miller is an Assistant Professor in the WVU School of Medicine Department of Disability Studies where she

teaches Disability and the Family to graduate and undergraduate students. She is a member of the Disability Studies Faculty Committee and was elected to serve on WVU Faculty Senate as a School of Medicine representative. Dr. Miller served as Project Manager during the previous cycle of the WV LEND grant and became familiar with WV LEND federal project officer and has been attending LEND Director Meetings. Dr. Miller works closely with previous WV LEND Director and now Co-Director Dr. Jaynes.

Co-Project Director

The Co-Principal Investigator/Project Director (FTE 15%), Dr. Maggie Jaynes, is a board-certified **pediatric neurologist** with thirty years experience serving children with autism, neurodevelopmental disabilities and other special health care concerns across. During the first decade of her career she taught children with learning disabilities then became the Diagnostician/IEP Coordinator for a county school system. Dr. Jaynes continues to be a **certified teacher**. As a physician, Dr. Jaynes has dedicated her practice to children with special health care needs. She is Professor Emeritus in the WVU School of Medicine. She serves on the Medical Advisory board of the WV CSHCN Program, on the Governor's Autism Trust Board and on the Mid-Atlantic Regional Telehealth Center Advisory Board as one of WV's representatives. Dr. Jaynes has been Program Director of HRSA funded grants including the Project Access Epilepsy Grant, which focused on innovative access to care through telehealth, and the WV State Implementation Grant. Dr. Jaynes serves on the National Institutes of Health (NIH)-funded Neurodevelopmental workgroup. She served as LEND Director for 24 years. She provides lectures to dental students and Disability Studies courses within the School of Medicine. Physical therapy and dental students, child psychiatry fellows, medical students and residents participate in her clinics. While Co-PD is Dr. Jaynes' primary role with WV LEND, she will also serve as **Education Mentor** and a secondary Medicine/Pediatric Provider Mentor.

Core Faculty/Interdisciplinary Program Leadership Team

The faculty of WV LEND consists of both Core Faculty and Supporting Faculty. The Core Faculty, including PD and co-PD, form the Interdisciplinary Program Leadership Team. This team of Core Faculty share responsibility for planning, designing, implementing, coordinating and evaluating WV LEND. Evaluation by Core Faculty include conducting CQI on all training and service elements within their discipline with emphasis on autism/DD. Core Faculty are responsible for the interdisciplinary curriculum of the training program for all trainees as demonstrated by their participation and direction of clinical and community-based, family-centered interdisciplinary clinics and programming. Core Faculty also provide mentorship to trainees within their discipline and also to related disciplines. For example, the WV LEND Occupational Therapy Mentor may often provide information and feedback to a physical therapy trainee. Core Faculty also help all LEND trainees, both long and short-term, view their disciplines from an autism/DD perspective. Core Faculty develop and participate in the trainee

recruitment/interview process, including criteria development and trainee recruitment for their specific discipline.

Core Faculty contribute to interdisciplinary leadership training, clinical mentorship, and academic activities. Non-clinical Core Faculty contribute to clinics by offering lived experience perspectives on autism/DD. The Core Faculty meet at least three times per year to assess progress towards meeting program goals (CQI), assess trainee progress, develop feedback for trainees and engage in formative program evaluation of the program (CQI). The Core Faculty represent their discipline both at an internal department or school level at WVU, with academic peers of the same discipline, and other professionals across WV and in the community.

The **Core Faculty/Interdisciplinary Program Leadership Team** represent the disciplines of **PD/Speech Language Pathology**, **co-PD, Occupational Therapy, Lived Experience (Family and Individual with Disabilities), and Medicine/Pediatric Provider.** This Core Faculty brings expertise in their discipline area and in their advanced studies and research interests. The following describes the Core Faculty:

Training Director and Occupational Therapy Mentor (FTE 50%), Amy Kurowski-Burt, EdD, MOT, OTR/L, is a board certified, registered, and licensed **occupational therapist**. Dr. Kurowski-Burt is a Full Professor and has a Doctor of Education (EdD) in Educational Psychology and a Masters of Occupational Therapy degree (MOT). She has 23 years of experience in occupational therapy, working with children and youth with autism/ DD in early intervention, schools, and outpatient settings. Dr. Kurowski-Burt worked in the Division of Occupational Therapy for 12 years as OT faculty and 2 years as the Master's Program Director. She continues to work alongside the division to mentor OT students during clinical experiences, as well as offer various lectures and lab activities on topics of expertise. Other work includes a variety of programs for children and young adults with autism/DD, such as the WVU Country Roads program, a college level program for young adults with intellectual and developmental disabilities. She has completed training on the Autism Diagnostic Observation Schedule (ADOS-2). Her areas of teaching and clinical expertise include autism, developmental disabilities, sensory integration, feeding, evaluation and assessment, treatment planning, and community participation, as well as interprofessional education and practice. Dr. Kurowski-Burt coordinated community training events for LEND and WVUCED, such as provider training on feeding, Mountain State Conference on Disabilities, and the Mind the Gap program, an autism support program for caregivers of children newly diagnosed with autism or are demonstrating red flags related to autism. She recently received AMCHP funding to develop a Train-the-Trainer program for pediatricians throughout WV focused on autism screening and diagnostics utilizing the STAT-MD, early identification model, developed by Vanderbilt University.

Family Mentor (contracted), Kristi DeLeurere, first came to WV LEND with lived disability experience as a trainee. After graduation from WV LEND, Kristi became the program's Family Mentor. Kristi has a son who was diagnosed with autism at age four. He was a micropremie who had an extended NICU stay resulting in tracheostomy and gastrostomy tube (g-tube) placement. Kristi's son is now an upcoming high school senior who still lives with a g-tube and is navigating the transition to adult medical providers and college. Kristi also has an older son who was diagnosed with ADHD in middle school and is currently enrolled in college. Kristi herself was recently diagnosed with autism as an adult. She is also a practicing **occupational therapist** within the WV Birth to Three Program where she is an instructor to other practitioners on the Early Start Denver Model (ESDM). Kristi also serves on the Feeding Matters Family-Centered Pediatric Feeding Research Consortium. Kristi has served as Family Mentor for two years.

Individual with a Disability Mentor (FTE 34%), Cheryl Childs, began her WV LEND journey as a trainee. After completing the WV LEND Program, Cheryl became a lived experience mentor. Cheryl is a wheelchair user who has experience transitioning from pediatric to adult medical services, moving far from family to attend college, parenting and entering the workforce. Cheryl served on a Lived Experience Panel at the WVU School of Medicine to support an AUCD ITAC (Interdisciplinary Technical Assistance Center on Autism and Developmental Disabilities) grant and is a guest speaker in the WV LEND Leadership Seminar series. Cheryl also works with the WVUCED Paths for Parents Program supporting children and families with special health care needs across WV.

The **Medicine/Pediatric Provider Mentor** (FTE 5%), David Huss, MD has more than 10 years of experience as a pediatric neurologist in neurodevelopmental disabilities at WVU Medicine. He is Medical Director of Neurological Services at the WVU Neurodevelopmental Center. He specializes in the areas of child development, neurodevelopmental disabilities, assessment of neurodevelopmental disorders, autism, epilepsy and telemedicine. He is an assistant professor in the WVU School of Medicine Department of Neurology. He has three board certifications: Psychiatry and Neurology, Neurology with Special Qualifications in Child Neurology; Pediatrics; and Psychiatry and Neurology, Neurodevelopmental Disabilities.

The Supporting Faculty represent the disciplines **Disability Studies, Community, Social Work**, **Physical Therapy,** and **Nutrition.** Supporting Faculty strengthen curriculum and expand mentorship into a greater number of disciplines than is present in the Core Faculty. Supporting Faculty have demonstrated expertise in the area of autism/DD and are well-respected by their colleagues. Supporting Faculty have successfully mentored former long, medium and short-term trainees. Each Supporting Faculty are licensed and credentialed as their disciplines require.

Disability Studies Mentor (FTE 10%), Shannon Zimmerman, is an Assistant Professor of Disability Studies within the WVU School of Medicine. She is the instructor for the program's

Disability and Policy and Capstone courses. Dr. Zimmerman serves as the data analyst for the WVUCED and as an instructor for the WVU Inclusive Post-Secondary Education Program, Country Roads. Dr. Zimmerman earned her PhD at WVU in Kinesiology specializing in adapted physical activity. Her research expertise includes mixed methods design to support program development and evaluation. In addition to her academic work, Dr. Zimmerman is actively involved in implementing adaptive sport programs that promote physical activity for individuals with disabilities and is a **certified teacher**. She is a member of the Disability Studies Faculty Committee and the Promotion and Tenure Committee for the WVU School of Medicine. Dr. Zimmerman will also support WV LEND through her role as WVUCED **data analyst** by mentoring trainees on research projects and providing oversight of project data organization.

The **Community Mentor** (contracted), Cheyann McQuain, MSW, is the program director at the Disability Action Center in Fairmont, WV. She is a recent WV LEND graduate. Cheyann has worked with adults with autism/DD for the past seven years. She will also serve as a secondary **Social Work Mentor**.

The **Curriculum Director** and **Social Work Mentor** (contracted), Jennifer Forrester, MSW, LCSW, has 30 years experience as a social worker. Jennifer began her career in Child Protective Services (CPS) then transitioned to the Klingberg Neurodevelopmental Center where she supported children and youth with autism/DD and their families through the evaluation and diagnostic process. Jennifer's work at Klingberg also included support to individuals and their families recently diagnosed with ADD/ADHD. Jennifer has a unique perspective as she experienced the progression of autism diagnosis and intervention from the early 90s through today. Jennifer taught Disability Studies courses for many years before transitioning to a community focus where she supports individuals with autism/DD in accessing essential services, including meals and medical care. She has served as Curriculum Director for the past 15 years.

The **Physical Therapy Mentor** (FTE 5%), Ketaki Inamdar, PT, PhD has 10 years experience as a certified **physical therapist**. She has a Bachelor's and Masters in Physical Therapy. She has Doctorate in Rehabilitation and Movement Sciences with a dissertation entitled "Tummy Time in Infancy: Real-World Assessments and Associations with Developmental Outcomes in Early Childhood." She is the Principal Investigator of CRADLE, Pediatric neuromotor assessment and functional rehabilitation lab at the WVU Health Sciences Center.

The **Nutrition Mentor** (FTE 67%), Kari Price, MS, RD, LD is a Registered Dietitian Nutritionist and WV Licensed Dietitian. She works as the dietitian for the WVUCED Feeding and Swallowing Clinic and mentors with LEND trainees. She has a BS/MS in Human Nutrition and Foods. She was part of a Maternal & Child Health LEAD Traineeship, a program designed to train individuals from various disciplines in working with children who have DD in the Appalachian area. Kari provides annual lectures for the WVU Disability Studies and WVU

Human Nutrition and Foods Departments. She also provides CEs on nutrition for children and youth with autism/DD for dieteticians through the Commission on Dietetic Registration.

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ATTACHMENT 1: WORK PLAN

WORK PLAN Yearly

Goal 1: ***Provide evidence-based interdisciplinary education and training that advances the understanding of children and youth with autism/DD across the lifespan and their families by increasing academic and clinical knowledge and building leadership skills for trainees and mentors including students, individuals with lived disability experience and practicing professionals.***

Objective	Activity	Person(s) Responsible	Measurement	Timeline		
				Aug-Dec	Jan-May	Jun-Jul
1-1: Recruit & retain 18 long-term trainees from 10 disciplines yearly.	Recruit through job fairs, individual disciplines; Trainees participate in recruitment	PD, CP, PM, TD, CF, LE, M, T	Number/discipline of trainees recruited	X		X
1-2: Recruit & retain 15 medium-term trainees	Trainee assisted recruitment, students rotating through CED	PD, PM, T, TD, CF, M,	Number of medium-term trainees recruited	X	X	X
1-3: Recruit & retain mentors from 9 disciplines with at least one lived experience and one family mentor.	Advertise to DD council and CED advisory committee	PD, CP, PM,TD	Number of mentors; number of mentor disciplines; completion of Mentor Agreement			X
1-4: Provide interdisciplinary experiential & academic training to long- and medium-term trainees.	ITP, Clinics; Leadership and community clinics; learning modules	PD,CP, PM, TD, CF, LE,M	Number & range of didactic; clinical experiential activities; trainee competencies met; mentor evaluation	X	X	
1-5: Provide 3 continuing education opportunities yearly for community and practicing professionals.	Learning modules; local, state & national presentations/webinars; Rural Collaborative activities	PD, CP, TD, CF, LE, M,	Number and type of recipients requesting CE; Training evaluations	X	X	X
1-6: Demonstrate trainees' knowledge of MCH concepts	ITP, Interdisciplinary clinical activities; Leadership seminars	PD, CP, M, TD, CF, LE	Pre-Post test scores; successful completion of requirements	X	X	
1-7: Provide trainees a curriculum to recognize & value seamless & accessible care specific to lived experience perspective	Leadership self-assessment; LEND Orientation;, Leadership Seminar; Interdisciplinary clinics, mentoring	PD, CP,TD, CF, LE, M	Self-reflection (ISR); ITP goals obtained; Lived Experience Mentor evaluation;	X	X	
1-8: Provide skills in systems development & standards of care.	Leadership Seminar, Trainee Tracks	PD, CP,TD, CF, M, LE	Successful completion of requirements per rubric (oral & Chat)	X	X	
1-9: Understand &discuss MCH focused legislation	Follow Bill presentation	PD, CP,TD, CF, LE, M	Presentation evaluation by self, peer and mentor; discussion rubric		X	
1-10: Increase by 10% long-term trainees reporting of engaging in policy activities.	Trainee activity in policy forums; completion of trainee surveys	PM, TD, CP, PM	Number trainees attending policy forums;Trainee Exit Interview; trainee follow-up survey		X	X

PD = Program Director; PM = Program Manager; TD = Training Director; CD = Research Mentor; CF = Core Faculty; LE = Lived Experience Mentors; M = Mentors; T=Trainees

PD= Program Director; CD= Co-Program Director; PM= Program Manager; TD= Training Director; CD= Research Mentor; CF= Core Faculty; LE= Lived Experience Mentors; M=Mentors; T=Trainees

Goal 2: ***Increase the identification and use of evidenced-based screening, diagnostics and treatment for children and youth with autism/DD across the lifespan by providing exemplary and high quality, family-centered and interdisciplinary collaborative academic and experiential training opportunities.***						
Objective	**Activity**	**Person(s) Responsible**	**Measurement**	**Timeline**		
				Aug-Dec	**Jan-May**	**Jun-Jul**
2-1: Provide 18 long- & 15 medium-term trainees knowledge and characteristics of children & youth with autism/DD.	SOLE assignments; Seminars, Attending Center and community clinics; pediatric resident rotation	PD, TD, CP, CF, LE, M	Individual self-reflection (ISR), number of clinic attended per trainee, number of training participants; SOLE completion	X	X	
2-2: Provide 4 observations of inter-disciplinary screening and diagnosis of autism/DD.	Neurodevelopmental Center observation and CED Clinics observations	PD, CP, TD CF, M	ISR; record of attendance in clinic logs	X	X	
2-3: Increase the number of practicing professionals skilled to screen & diagnose autism/DD.	Continuing education Technical assistance	PD, CP,TD, CF, M	Number of participants; documented skill acquisition;training evaluations	X	X	X
2-4: Provide trainees the knowledge of evidence-based intervention of autism/DD.	Didactic content, interdisciplinary training in Autism clinic	PD, CP, TD, CF, M, LE	Attendance logs for clinics, Pre/post tests of skill acquisition	X	X	
2-5: Provide 4 interdisciplinary experiences supporting high quality, needs based approach to newly diagnosed individuals with autism/DD.	Participation in LEND Support Program; Participation in Feeding Clinic	PD, CP, TD, CF, LE, M	Attendance log; number and quality of Individualized Action Plans (IAP) & parent satisfaction with IAP; LE mentor evaluation	X	X	X
2-6: Provide trainees with 4 interactive experiences demonstrating evidence-based intervention	Neurodevelopmental Center, community clinics	PD, CP, TD, CF, M	Attendance log; ISR	X	X	
2-7: Increase capacity of rural systems of care to use innovative practice models to expand diagnostic and treatment services through technical assistance activities.	Activities with LEND Rural Collaborative; Coaching sessions with community providers; Support activities with school system	PD, CP,TD, CF, M	Attendance hours; mentor evaluation # participants; #of activities; training eval	X	X	X

Goal 3: ***Develop and enhance innovative practice models that strengthen systems of care for individuals with autism/DD across the lifespan in rural communities built through partnerships among disciplines across West Virginia University (WVU), WVU Medicine and other state medical systems of care, community, state, regional and national entities.***

Objective	Activity	Person(s) Responsible	Measurement	Timeline		
				YR1	YR3	YR5
3-1: Provide trainees with a framework to build skills necessary to lead family centered interdisciplinary processes.	Attend Child Development Neurodevelopmental Clinic, Feeding and Swallowing Clinic, Complex Care Clinic	PD, TD, CF LE, M,	Pre- and Post-tests, documented participation in service models, Individual Self Reflection; Lived Experience Mentor evaluation rubric	X	X	
3-2: Ensure trainees demonstrate competency for varying perspectives in tandem with leadership skills to reflect an understanding of rural community health factors	Appalachian culture seminar; Completion of online modules, Participation in interdisciplinary clinics	PD, CP, TD, CF. LE, M	Mentor observations of performance and clinic discussions utilizing discussion rubric, Individual Self Reflection	X	X	
3-3: Develop or enhance partnerships across the state and with regional/national LENDs to demonstrate and promote evidence-based solutions to the needs of rural and children with autism/DD.	Observation of WV Medicine Pediatric telemedicine program, WVU system, Title V, MCH and regional LENDS; attending the Rural LEND Collaborative activities;	PD, CP, TD,	# of meetings attended; sharing of results of meetings; products developed and shared	X	X	X
3-4: Provide trainees with 5 opportunities to collaborate with Title V and other MCH programs through service, training, CE, TA, product development and research.	Develop service training, CE, TA, product development, or research activities	PD, CP, TD CF	Products produced, dissemination/NCC exit interview,	X	X	
3-5: Provide trainees and mentors with 4 opportunities to engage in innovative service models to children and youth with autism/DD.	Feeding and Swallowing clinic, Telehealth clinics, Neurodevelopmental Center, Complex Care clinic	PD, CF, TD,	Documented attendance through ISR	X	X	X
3-6: Provide consultation, TA, dissemination of information and continuing education activities to 200 practicing professionals.	Technical assistance, dissemination of information, CE activities	PD, CP, TD, CF, M, LE	# of activities,# and range of products disseminated Training surveys	X	X	X

Goal 4: ***Increase the expertise of trainees and practicing professionals from local and state agencies, community organizations, and private providers to recognize the complex lifespan needs of children and youth with autism/DD with focus on the lived experience perspective to improve rural systems of care.***

Objective	Activity	Person(s) Responsible	Measurement	Timeline		
				YR1	YR3	YR5

4-1Provide trainees with the conceptual knowledge and performance skills to participate in and contribute to interdisciplinary team process in rural systems of care.	Feeding & Swallowing Clinic, LEND Support, Title V Clinics, Neurodevelopmental Clinics	PD, CP,TD, CF, LE, M	Pre- and Post-tests, ISR, Mentor observation per rubric	X	X	
4-2: Support trainees to plan, design and present research/badge projects focused on the lifespan care of individuals with autism/DD.	Badge/research project Presentation of Badge/research project	PD,TD, CD, CF, M	Completion of project Presentation of project with peer and mentor evaluation through rubric	X	X	
4-3: Provide opportunities for trainees to demonstrate ability to translate evidenced-based research into practice by linking academic and experiential forums.	Feeding Telehealth Clinics, community-based activities and interdisciplinary clinics Lead Journal Club	PD, TD, CF CD, M	Peer and Mentor observation through rubric: ISR	X	X	X
4-4: Provide opportunities for participation in 4 innovative model service delivery experiences that demonstrate family-centered interdisciplinary approaches to care.	Attending Feeding Clinic, Attending activities at the community partner DAC;	PD, TD, CF,	Mentor including LE mentors observation through rubric; ISR	X	X	X
4-5: Prepare individuals with autism/DD & their families to improve systems of care by using an experiential model to demonstrate self-directed care coordination, shared decision making &	CED clinics, Child Development and Neurodevelopmental clinics,	PD, TD, CF, LE	LE Mentor observation rubric, ISR	X	X	X
4-6: Increase by 10% the number of long-term trainees who work in an interdisciplinary manner to serve the MCH population.	Completion of long-term trainee survey	TD,PM, CF, T	NIRS data from long-term trainee surey		X	

ATTACHMENT 2: STAFFING PLAN/JOB DESCRIPTIONS FOR KEY PERSONNEL

WV LEND will be staffed by a Project Director (PD), co-Project Director (co-PD), Program Manager (PM), Training Director (TD), Curriculum Director (CD), and Core Faculty/Interdisciplinary Program Leadership Team who represent a full range of disciplines, including an individual with a disability and a family member of a person with a disability, well versed in the areas of autism/DD. The amount of time requested for each professional is based on contributions beyond the course of their discipline/school requirements. Funding is provided for services that are related to LEND and are not considered a part of their part of departmental requirements. Faculty representing academic departments are chosen by their department chairs to participate in WV LEND as faculty. Academic faculty may have a joint appointment in the Disabilities Studies Department in the WVU School of Medicine. The LEND Organizational Chart is found in Attachment 4.

Additional support staff/administrators are funded to assist in daily organizational activities. These would include secretary, financial staff, data support staff, dissemination staff and web/social medical support. Funding is dependent on anticipated time spent in support activities related to WV LEND.

Each Core Faculty/Interdisciplinary Program Leadership Team member and the PD, co-PD, TD, and CD have extensive experience in the support, screening, evaluation, diagnosis, assessment, and intervention of children and youth with autism/DD and those who are for them. The following are members of WV LEND Core Faculty. The titles, roles and responsibilities follow:

PROJECT DIRECTOR – responsible for and directs health professional leadership training project at the WVU Center for Excellence and Disabilities (WVUCED). The incumbent performs demonstrated and supervisory work and has primary responsibility for planning, coordinating and implementing the goals and activities of the project. The Project Director is responsible for the administrative functions required to operate the project. The incumbent serves as the primary liaison between WV LEND Program and their professional associates, academic affiliates, clinical departments, and discipline counterparts in the community, state, region, and across the country. The Project Director oversees the recruitment, selection, and retention of WV LEND trainees and mentors. The incumbent is administratively responsible to the WVUCED Director and academically responsible to the chairperson of their academic department.

CO-PROJECT DIRECTOR – responsible to support the Project Director in planning, coordinating and implementing the goals and activities of the project. The Co-Project Director is also an active liaison between WV LEND Program and their professional associates, academic affiliates, clinical departments, and discipline counterparts in the community, state, region, and across the country. The Co-Project Director is administratively responsible to the WVUCED Director and academically responsible to the chairperson of their academic department.

PROGRAM MANAGER –is responsible to assist the Project Director in planning, coordinating and implementing the goals and activities of the program. This position also assists the Program Director in daily administrative functions required to operate the project with 50% FTE dedicated to data management. The incumbent is administratively responsible to WV LEND Director and is academically responsible to the chairperson of his or her academic department. They work closely with the WVUCED Data Analyst and provide daily structure to the activities of the administrative support personnel.

TRAINING DIRECTOR - Under the administrative direction of WV LEND Project Director, the incumbent is responsible for coordinating and participating in the planning, development, organization and implementation of the interdisciplinary training activities of WV LEND. They define the criteria for recruitment of trainees. The Training Director works cooperatively with the project's Core Faculty in the recruitment, retention and evaluation of trainees. The Training Director works with WV LEND Core Faculty to review and evaluate programing and serves a mentor to trainees.

CURRICULUM DIRECTOR – Under the administrative direction of the LEND Project Director, the incumbent is responsible for coordinating and participating in the planning, developing, organizing and implementation of the community interdisciplinary training activities of WV LEND Program. The incumbent serves as a liaison between community projects, clinical sites and LEND. The incumbent is a mentor to trainees.

CORE FACULTY/INTERDISCPLINARY PROGRAM LEADERSHIP TEAM (PD, co-PD, TD, CD, PM, Medicine/Pediatric Provider Mentor, Lived Experience Mentors) – Under administrative direction of the LEND Project Director, the incumbent plans, develops and conducts the activities of the discipline components for the leadership training of all trainees. The incumbents serve as primary liaisons between the program and their professional associates, academic affiliates, clinical departments, and discipline counterparts in the community, state, region, and nationally. They define criteria for recruitment of trainees of their discipline and participate in the selection/interview committee. They review and evaluate LEND trainees and review and evaluate LEND programing ensuring that the program is working towards fulfilling goals and objectives. They ensure that the program is adhering to MCH principles.

SUPPORT FACULTY (Social Work, Nutrition, Physical Therapy, Disability Studies, Community Mentors) – Under the administrative direction of the LEND Project Director, the incumbent provides mentorship to all trainees and expands curriculum. Support Faculty also engage practicing professionals in CE in their discipline areas.

ADMINSTRATIVE SUPPORT - Under the administrative direction of the LEND Project Director, the incumbent provides organizational and scheduling support. The administrator also serves as an initial point of contact for trainee communications and social media.

Attachment 3 – Agreements with other Entities

Sample letters are provided.

Additional letters are available for review:

1. Lancaster – Chair of WVU Pediatrics
2. Brewer – Interim Dean of the WVU College of Law
3. Meigh – Interim Chair, Division of Communication Sciences and Disorders
4. Mulligan – Associate Professor and Chair, Physical Therapy
5. Statement of Collaboration – LEND Rural Collaborative
6. Huss- Medical Director of the WVU Neurodevelopmental Center

WestVirginiaUniversity
School of Medicine

June 30, 2026

Dear Dr. Miller,

I am writing in support of your grant proposal "*Leadership Education in Neurodevelopmental Disabilities and Other Related Disabilities*" (LEND) and its goal of improving the health status of infants, children and adults living in rural areas who have or are at risk for autism and other developmental disabilities. This program has had a relationship with the WVU Physician Assistant (PA) Program for the last three years and helps to support its mission. The physician assistant trainees have found their LEND training to be valuable and to enhance their physician assistant education with advanced disability leadership training and hands-on learning opportunities.

The WVU PA program is very willing to have its faculty provide mentorship, clinical expertise and didactic learning to trainees in the WVU LEND program as part of their clinical and educational mission. In turn the WVU LEND mentors will provide education and interdisciplinary clinical experiences to the physician assistant students. The WVU LEND Program will fund at least one PA trainee per year.

The Physician Assistant Program is willing to share recruitment information with their students as well as support PA trainees with scheduling to allow for in-person engagement when possible.

Thank you for continuing the long-standing relationship between the WVU Physician Assistant Program and the WVU LEND Program.

Sincerely,

Jennifer Momen, MD, MPH, FAAP
Associate Professor and Program Director
Division of Physician Assistant Studies
WVU School of Medicine
jjmomen@hsc.wvu.edu
304-293-5466

Department of Human Performance
Division of Physician Assistant Studies
PO BOX 9226
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PH: 304-293-4349 Fax: 304-293-7105

West Virginia University is an Equal Opportunity Employer.

WestVirginiaUniversity.
SCHOOL OF MEDICINE

June 30, 2026

Dear Dr. Miller,

I am writing in support of your grant proposal, "Leadership Education in Neurodevelopmental Disabilities and Other Related Disabilities" (LEND), and its goal of improving the health status of infants, children, and adults living in rural areas who have or are at risk for autism and other developmental disabilities. This program has a long-standing relationship with the WVU Physical Therapy (PT) Program and supports its mission. Physical therapy trainees have found their LEND training valuable, enhancing their education through advanced disability leadership training and hands-on learning opportunities.

The WVU PT program is eager to have its faculty provide mentorship, clinical expertise, and didactic instruction to trainees in the WVU LEND program as part of its clinical and educational mission. In turn, the WVU LEND mentors will provide education and interdisciplinary clinical experiences to physical therapy students. The WVU LEND Program will fund at least one PT trainee per academic year and Dr. Ketaki Inamdar, PT, PhD, for 5% FTE.

The Division of Physical Therapy is willing to share recruitment information with its students and to support PT trainees with scheduling to enable in-person engagement when possible.

Thank you for continuing the established relationship between the WVU Division of Physical Therapy and the WVU LEND Program.

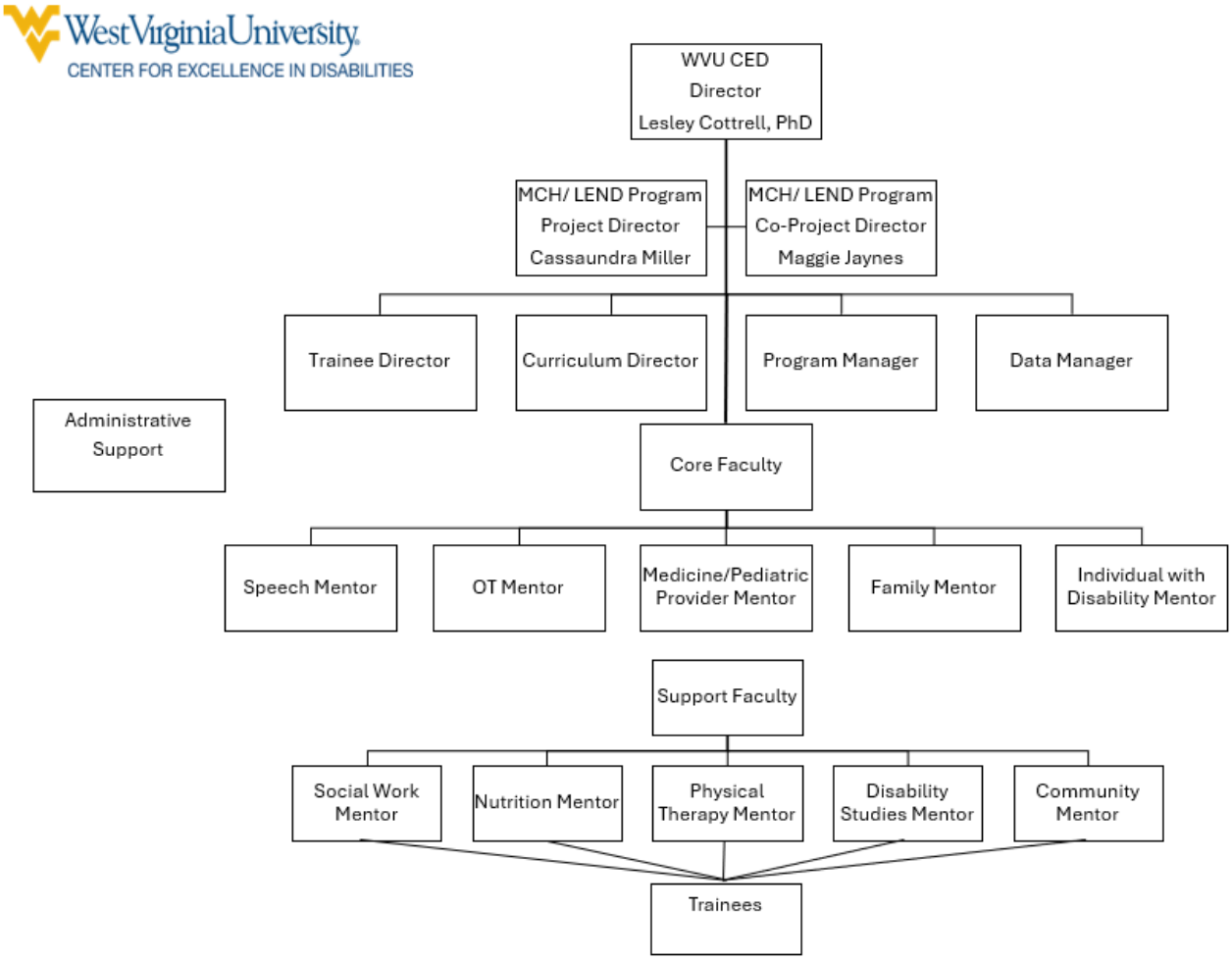
Sincerely,

Ivan Mulligan PT, DSc, ATC, CSCS
Board Certified Clinical Specialist in Sports Physical Therapy
Associate Professor and Chair

DEPARTMENT OF HUMAN PERFORMANCE, PHYSICAL THERAPY DIVISION
1 Medical Center Drive | PO BOX 9226
Morgantown, WV 26505
O 304-293-3610 F 304-293-7105

WVU is an EEO/Affirmative Action Employer — Minority/Female/Disability/Veteran.

Attachment 4: Project Organization Chart



ATTACHMENT 5: TABLES, CHARTS, SYLLABUS/CURRICULUM

WV LEND CURRICULUM

Maternal Child Health (MCH) Leadership Competencies
MCH Knowledge Base/ Context; Self-Reflection; Ethics; Critical Thinking; Communication; Negotiation & Conflict Resolution; Community Health Factors; Lived Experience in MCH; Teaching, Coaching & Mentoring; Interdisciplinary/ Interprofessional Team Building; Systems Thinking; Policy

Leadership

WHAT WE DO

- Interdisciplinary Leadership
- Family-Centered Care & Family Partnerships
- Cultural & Linguistic Competence
- Communication Skills
- Autism/ DD Supports
- Systems Thinking

HOW WE DO IT/ACHIEVE IT

- Individualized Training Plans
- Mentorship
- Family Engagement Activities
- Leadership Seminars
- e- Portfolio

Interdisciplinary Training

WHAT WE DO

- Interdisciplinary Clinic Attendance
- Feeding & Swallowing Clinic
- LEND Support Program
- Community & Clinical Observational Experiences
- Neurodevelopmental Center Autism Assessments and Interventions

HOW WE DO IT/ACHIEVE IT

- In-vivo experience as clinician on Interdisciplinary Team
- Interdisciplinary Staff Meetings
- Discipline report writing
- Interdisciplinary academic discussions

Academic

WHAT WE DO

- Trainee Tracks
- Introductory Disability Institute
- Disability Identity Badge
- Microlearning Sessions
- Training for Autism screening, diagnostics
- Following Bill
- Lived Experience Activities

HOW WE DO IT/ACHIEVE IT

- Required readings
- Didactic seminars from experts in respective topics
- Independent learning modules
- Discussions

Research

WHAT WE DO

- Journal Club
- Research trainee track
- Research activities with mentor
- Badge Project
- Conference presentations

HOW WE DO IT/ACHIEVE IT

- Presentation for Journal Club
- Research or Capstone Project
- Dissemination opportunities

Disciplinary/Clinical Experience
Current disciplines: Speech-Language Pathology, Family, Individual with a Disability, Occupational Therapy, Psychology, Social Work, Physical Therapy, Nutrition,, Neurodevelopmental Disabilities, Pediatric Neurology, Special Education, Disability Studies.

Syllabi for Curriculum Forums/Practicum Settings

LEND Expo Orientation

Purpose: This activity serves as an orientation to LEND and introduction to LEND activities. We will review the WV Trainee Handbook, discuss and develop Individual Training Plan (ITP) with mentors; introduce LEND trainee expectations and begin engagement with curricular components; team building exercises; introduction to general disability education topics; introduction to disability-related community activities; preparation of LEND projects.

Method: In person meeting including independent activities, didactic learning, group interactions, distribution of materials, group/interdisciplinary activities

Content: Participation timeline and required activities introduction and discussion; WV LEND policies and procedures; discussion of research and disability topics; preparation of activities such as journal club, research, badge project; team building activities; LEND national and state activity opportunities; meet LEND mentors; develop ITP

Time Commitment: Two days

Method of Evaluation: ITP, mentor feedback, reflective activities, completion of required preparatory LEND tasks

Introductory Disability Institute

Purpose: Establish a strong background on foundational concepts

Method: Online, asynchronous learning activities with biweekly discussion meetings as a cohort; and meetings with their LEND mentors

Content: Trainees will complete six modules. These include reviewing the AUCD LEND Trainee Orientation Modules, the Interactive MCH Timeline, and reflecting where they are and where they hope to be after completing the LEND program. Trainees will also learn about models, theories, and frameworks for disability leadership; transition supports throughout the lifespan, and complete interactive activities related to neurodevelopment and autism. They will also complete an introductory module on research, evidence-based practice, interprofessional education, and educational policy. Trainees will complete a self-assessment using the MCH Navigator Tool which will help track needs for Leadership Competencies, develop their Individual Training Plan, and learn about their Clifton Strengths

Time Commitment: 30 hours of asynchronous content, 3 hours for cohort meetings, 1 hour for mentor meeting

Method of Evaluation: IPE pre assessment comparison, MCH leadership competencies, self reflection, knowledge checks

LEND Trainee Tracks

Purpose: Provide specific information learning and experiential activities to trainees beyond general required WV LEND activities. All trainees will select one major track and at least one minor track to complete.

Method: Online and face-to-face sessions, synchronous and asynchronous activities, community-based activities and didactic sessions

Content: Options for trainee tracks will include the following: advanced disability leadership, interprofessional collaboration, research, clinical with emphasis on rural care, autism diagnostics and supports, and MCH-focused legislation and improving systems of care

Time Commitment: Major track is 40 hours; minor track is 20 hours

Method of Evaluation: Completion of track activities, self reflection, mentor meetings and feedback

Family Engagement Activities

Purpose: Develop family-centered leaders who appreciate and improve systems of care for families through a hands-on demonstration of the impact of disability on the child, family and community, learn about coordination of medical care, understand the impact of disability on family, understand resources available in WV and surrounding states

Method: Experiential small group interactions with designated families including nonprofessional interactions such as conducting a project with the family based on needs

Content: Family-centered care, community resources, medical home model, role of family in directing services, environmental and social influences, effects on family, transitions

Time Commitment: 10-20 hours

Method of Evaluation: Preparation educational activities, record of project with family, experience reflection

Leadership Seminar

Purpose: To prepare trainees for disability informed and advanced leadership roles focusing on interdisciplinary practice, understanding and using research, develop the ability to think critically about MCH issues at all levels, respond to changing community supports for people with disabilities particularly autism, contribute to ongoing recruitment and training of MCH leaders

Method: Didactic seminar with varied interactive experiences; expert guided discussions

Content: Based on interest assessment completed by trainees prior to start of the semester; examples of topics include rural healthcare, community supports, neurodevelopmental assessment and intervention for people with autism, family centered collaborative care, interprofessional collaboration, and transitions throughout the lifespan

Time Commitment: 2 hours each week

Method of Evaluation: Attendance logs; exit interviews; pre- and post-tests; mentor feedback; online learning activities

Microlearning Sessions

Purpose: Understand and apply various disability-related concepts, current issues in disabilities, and autism-related practice and community supports

Method: Interactive, didactic sessions

Content: Sessions will be focused on concepts in the following categories: academic, clinical, research, and support for community and family. Some specific examples are: nutrition, disability current events, transitions, autism screening, developmental screenings, caregiver support programs and strategies, transitions, inclusive practice in research

Time Commitment: 20 minutes per session, a minimum of 4 sessions per semester

Method of Evaluation: Self reflection, learning activity completion, leading sessions

Feeding and Swallowing Clinic

Purpose: Understands the needs of families and children with autism and other neurodevelopmental disabilities with feeding/nutrition issues, demonstrate critical thinking skills related to feeding challenges, discuss strategies and supports for families and children with an interdisciplinary team, understand the impact on child and family

Method: Weekly interdisciplinary chart review (prestaffing), evaluation clinic, poststaffing

Content: Medical, social, cultural, developmental, and behavioral concepts related to assessment and intervention planning on an interdisciplinary team with patient- and family-centered, strengths-based care

Time Commitment: At least 2 chart reviews and 2 evaluation sessions

Method of Evaluation: Mentor feedback, self reflection, participation and engagement with caregivers and family

Disability Identity Badge and Project

Purpose: Develop awareness of education, research, policy and systems of care foundation for individuals with all abilities, understand the disability experience

Method: Online, asynchronous learning activities, project development and dissemination

Content: Disability experiences and viewpoints, accessible language, accessible documents, family-centered practice, models of disability, community-focused badge project

Time Commitment: At least 40 hours

Method of Evaluation: Meetings with mentor, completion of learning activities, self reflection, badge project development and dissemination

Community Support Education Fairs

Purpose: To engage with community members throughout West Virginia providing disability-related education and activities for increased disability awareness and community support awareness and become disability leaders.

Method: On-site community education booths and activities

Content: Elementary, middle and high school disability fair with Monongalia County Schools; Spread the Word campaign, various pop-up community events throughout West Virginia

Time Commitment: 6-8 hours

Method of Evaluation: Participation in event, mentor feedback, self reflection, community feedback

Reading for Reflection and Growth

Purpose: Build awareness of disabilities and other MCH populations, enhance leadership skills

Method: Self-paced reading, group discussion and reflection

Content: Disability focused books, books written by an author with a disability, rural healthcare and Appalachian culture, autism experiences, MCH populations' experiences, leadership-focused books

Time Commitment: Variable

Method of Evaluation: Reflective activities, knowledge checks, group discussion, book clubs

Following Bill

Purpose: This longitudinal policy activity supports trainees in understanding United States bills focused on services and supports for individuals with disabilities and their families, discuss the progression of the bill, differentiate various bills

Method: Didactic sessions, group discussion, presentations

Content: Disability-focused and MCH population-focused legislation and process

Time Commitment: 1 hour per month

Method of Evaluation: Peer feedback, mentor feedback, learning activities

Lived Experience Activities

Purpose: Understand the perspectives of people with disabilities and their families/ caregivers

Method: Synchronous and asynchronous learning activities, didactic sessions, interdisciplinary group discussions, clinical activities

Content: Live sessions with people with lived experience, recorded talks, podcasts, MCH People with Lived Experience Learning Bundle

Time Commitment: At least 10 hours

Method of Evaluation: Self reflection, learning activities

Research and Journal Club

Purpose: Introduce research including methodology, critical review skills, varied designs, interdisciplinary populations, translation into practice, develop skills to critically evaluate and utilize research

Method: Group discussions, didactic sessions, mentoring, interactions with the Rural Collaborative LEND group, synchronous and asynchronous online sessions, featured journal article station

Content: Evidence-based, peer reviewed research as it applies to MCH populations, WVU only and rural workgroup monthly journal clubs

Time Commitment: 4 hours monthly, individual participation as needed

Method of Evaluation: participation in journal clubs, leadership of journal club discussions, critical appraisal worksheets

Reflective Practice Activities

Purpose: Develop self-reflection skills to enhance leadership growth; document and reflect on experiences regarding self, others and the community; build written skills

Method: Experiential community-based activities, discussions with mentors, learning activities

Content: MCH Leadership Competencies, how reflection leads to personal and professional growth, reflective practice activities, e-portfolio

Time Commitment: Occurs as part of all activities

Method of Evaluation: Mentor feedback, learning activities, peer feedback, e-portfolio, ITP review, exit interview (NOTE: parts of the reflective activities may be private and only shared as permitted by trainee)

Other Disability-Focused Learning Activities

Purpose: Understand current issues related to disabilities

Method: Synchronous or asynchronous online sessions, in person events

Content: Issues across many speciality areas in pediatric healthcare, focused on disability service and developing areas of disability practice, autism specific community supports and evidence-based practice, policy and disability

Time Commitment: varied

Method of Evaluation: Reflection learning activity

Experience Options:

- WVU CED Feeding ECHO
- WVU Pediatric Grand Rounds
- WVU CED Grand Rounds
- Disability-focused podcasts
- Disability-focused conferences: West Virginia Mountain State Conference, AUCD Annual Conference, AUCD Policy Conference, discipline specific state and national conferences
- CED Passport Activity; trainees learn about and engage with all WVU CED programs and clinics
- Disability and leadership focused pop-up activities; these are activities that are announced throughout the training year which trainees could benefit from such as workshops, symposiums, community events, etc.

CED In-House Clinics and Opportunities

Purpose: To build interdisciplinary clinical skills, understand various community supports available for people with disabilities, especially autism, and complex medical conditions and their families, understand various strategies utilized to increase quality of life for people with disabilities and their families, observe evidence-based practice and evaluation, understand transition supports available for people with disabilities specifically autism, disseminate resources to support families, understand barriers to obtaining services for autism/DD

Method: Modeling, group discussion, learning activities, evaluation/treatment with mentor guidance, interdisciplinary conversation including the child and family

Content: Introduction to interdisciplinary evaluation/treatment, systems of care, family-centered practice, transition-focused interventions and supports, disability focused clinical care, behavior, feeding and swallowing, nutrition, caregiver and disability-focused support groups, strengths-based approaches, trauma-informed approaches, assistive technology

Time Commitment: At least two clinics; individual participation will vary based on ITP

Method of Evaluation: Mentor feedback, exit interview, learning activities, participation and engagement, self-reflection

Experience Options:

- LEND Support Program – Once monthly sessions staffed by LEND trainees and interdisciplinary mentors/staff to address concerns of children with autism and other neurodevelopmental disabilities and their families.
- Assistive Technology (AT) Training – Engagement with the West Virginia Assistive Technology System (WVATS), interacting with varied AT, completion of assistive technology-focused online trainings.
- Country Roads – Service as a peer mentor for students with intellectual and developmental disabilities enrolled in the West Virginia University Country Roads Inclusive Postsecondary Education program (IPSE). Observe and support Country Roads courses, support students during study halls.
- Disability Employment – Observe disability employment assessments and technical assistance programming.
- Pediatric and Adolescent Occupational Therapy – Weekly occupational therapy evaluations and therapy sessions for children with occupational therapy-related needs.
- Positive Behavior Support (PBS) – Observe the PBS team completing evidence-based strategies for families and children with challenging behaviors to improve quality of life, completion of behavior-focused online trainings.
- Traumatic Brain Injury (TBI) – Observe TBI-focused support groups, participate in TBI training activities.

Community-Based Clinics and Services

Purpose: Observe interprofessional community-based practice, build disability service skills, understand typical and atypical pediatric development, evidence-based treatment for children with autism, understand transition supports and interdisciplinary aspects of transition available for people with disabilities specifically autism, understand resources and supports for children

and youth and their families, including educational, legal, and healthcare supports, and differing needs of families.

Method: Modeling by mentors, guided trainee service provision and follow-up, experiential learning, group discussions, self reflection

Content: Systems of care, family-centered care, medical home model, Appalachian (rural) culture and underserved populations, evidence-based practice, Title V programs, observation of assessments

Time Commitment: At least two clinics; individual participation will vary based on ITP

Method of Evaluation: Mentor feedback, exit interview, learning activities, participation and engagement, self-reflection

Clinical Practicum and Field Site Opportunities:

WVU Medicine Golisano Children's Hospital

- Cerebral Palsy and Spasticity Clinic
- Complex Care Clinic
- Music Therapy

WVU Medicine Golisano Children's Neurodevelopmental Center

- Applied Behavior Analysis (ABA) Therapy
- Early Learner Program
- Physical Therapy
- Occupational Therapy
- Speech Therapy
- Developmental assessments
- Autism diagnostic evaluations

WVU Faculty Practice Clinics

- WVU Speech, Language and Hearing Clinic, speciality focuses on augmentative communication and audiology
- WVU Physical Therapy and Occupational Therapy Clinic
- CRADLE (Pediatric neuromotor assessment and functional rehabilitation) Lab
- WVU Child Learning Center
- WVU Office of Student Accommodations Autism Support Program

Other sites

- Key Learning Autism Center
- West Virginia Birth to Three
- Stepping Stones
- LiveAbility
- PACE Enterprises
- Disability Action Center (DAC)

Disability Immersion Program for Medium-Term Trainees

Required topics and activities (32 hours):	Badge Mentorship Meetings
	Reflection: Viewpoints of Disability
	Crip Camp Documentary
	Person-Centered Language
	Accessible Documents
	Family Goals vs. Professional Goals
	Disability 101 Webinar
	Models of Disability
	Neurodiversity
	Badge Project and Reflection
Flex options for additional (8 hours):	HIPPA Training
	Journal Club
	CED Program Exploration
	Feeding Clinic
	ADA learning activity
	Grand Rounds
	Journal Article Critical Analysis
	Other self directed activity

TRAINEE LEADERSHIP COMPETENCIES BY CURRICULUM FORUMS

SI-Summer Institute,AC=Academic Content, FE=Family Engagement, LS=Leadership seminar, R= Research, JC=Journal Club, CE=Clinical Experiences; CB=Community-Based Experiences, SR= Self-Reflection, IA=Interdisciplinary Activities

1: MCH Knowledge Base/Context *– Promotion of the health and well-being of women, children, adolescents/young adults, and families, and children with special health care needs; a focus on individuals, families, communities, populations and systems of care; a life course perspective as an organizing framework. MCH leaders will:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
1-1: Describe MCH populations, history and current structure of the key MCH programs, including Title V.	x	x		x	x	x	x			x
1-2: Identify MCH program services, limitations and gaps.	x	x		x	x	x	x			x
1-3: Articulate the key policies and laws that affect MCH populations.	x	x		x	x	x	x			x
1-4: Identify principles of public health and population data collection and analysis.	x	x		x	x	x	x			x
1-5:understand value of partnering with people with relevant lived experience to improve programs, policies	x	x		x	x	x	x	x		x
1-6: Ensure community factors are at the forefront of program planning & service delivery	x	x		x	x	x	x	x		x
2: Self-reflection *– Process of assessing the impact of personal values, beliefs, communication styles, cultural influences, and experiences on one's personal and professional leadership style. MCH leaders reflect on:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
2-1: Deeper understanding of experiences, values &beliefs & how these may influence action & learning	x	x	x	x	x	x			x	
2-2: Personal strengths in both informal and organizational contexts.	x	x	x	x	x	x			x	
2-3: Personal leadership styles and attributes in relation to work settings.	x	x	x	x	x	x			x	
2-4: Impact of self-assessment & self-reflection on leadership style	x	x	x	x	x	x			x	
3: Ethics *– Ethical behavior in professional roles includes conduct congruent with accepted principles and values. MCH leaders will demonstrate:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
3-1: The ethical and legal principles, values, and behaviors (such as beneficence, non-maleficence, truthfulness, justice, and respect for autonomy) underlying professional conduct within community, health care, and public health settings.		x		x	x	x				x
3-2: Their professional association's code of ethics.		x		x	x	x				x
3-3: Institutional review board processes and criteria for ensuring ethical study design and informed consent in human subject research and translations of research to practice, focusing on individuals with disabilities.		x		x	x	x				x
4: Critical thinking *– The ability to identify an issue or problem, frame it as a specific question, consider it from multiple perspectives, evaluate relevant information, and develop a reasoned resolution. Evidence-based decision-making and Implementation science are vital components of critical thinking in order to promote the adoption and integration of evidence-based practices, interventions, and policies. MCH leaders will demonstrate:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
4-1: The cognitive hierarchy of critical thinking; knowledge, comprehension of, application, analysis, synthesis, and evaluation.				x	x	x				x
4-2: Basic statistics, epidemiology, qualitative and quantitative research, systematic review, and meta-analysis.				x	x	x				x
4-3: The levels of evidence used in guidelines and recommendations of their professional organizations.				x	x	x				x

5: Communication *– The verbal, nonverbal, and written sharing of information. Skillful communication is the ability to convey information to and receive information from others effectively. MCH leaders will demonstrate:*	**SI**	**AC**	**FE**	**LS**	**R**	**JJC**	**CE**	**CB**	**SR**	**IA**
5-1: Principles of communication for all modalities, including verbal, written and nonverbal, in various practice, policy, and research settings.	x	x	x	x	x	x	x	x	x	x
5-2: Approaches to overcome communication challenges, such as literacy levels, disability, cultural meanings, language differences, professional terms, and acronyms.	x	x	x	x	x	x	x	x	x	x
6: Negotiation and conflict resolution *– Knowledge and skills in negotiation and conflict resolution to address differences among all those included with the development and delivery of health care issues. MCH leaders will demonstrate:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
6-1: Characteristics, manifestations and sources of conflict in organizational contexts.				x	x	x				x
6-2: The theories pertaining to conflict management and negotiation among groups with differing interests.				x	x	x				x
6-3: The strategies and techniques useful for successful negotiation with various groups.				x	x	x				x
6-4: The potentially positive/catalyst role of conflict in the change process.				x	x	x				x
7: Community Health Factors*– MCH professionals will understand how complex community factors influence the health of women, children families.Community input is invaluable in the effective design of intervention programs and research studies. MCH leaders will demonstrate the knowledge of:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
7-1: Community factors can influence& interact to influence health &access to health care services.	x	x	x	x	x	x	x	x		x
7-2:Addressing community factors can positively impact MCH outcomes.	x	x	x	x	x	x	x	x		x
7-3: Id & elevate strengths of individuals and communities that value people with a variety of backgrounds.	x	x	x	x	x	x	x	x		x
7-4: Incorporate understanding & appreciation of differences in perspectives into professional behaviors.	x	x	x	x	x	x	x	x		x
7-5: Use data-driven tool & use plain language to present data.	x	x	x	x	x	x	x	x		x
8: Lived Experience in MCH*– Ensuring the health and well-being of children, including those with special health care needs, and their families through respectful family-professional collaboration and shared decision-making. Family and personal expertise is a body of knowledge that constitutes a discipline. MCH leaders while demonstrate a working knowledge of the:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
8-1: Definition of family-professional partnerships and the origin of the family-centered care perspective at the individual, organizational, and systems level.			x	x	x x	x	x	x		
8-2: The principles of family-centered care in MCH policies, programs, or clinical practice.			x	x		x	x	x		
9: Developing others through teaching, coaching and mentoring *- Primary strategies used to develop others are teaching, coaching, and mentoring. MCH leaders will demonstrate a working knowledge of:*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**
9-1: Teaching strategies and tools appropriate to the goals, context, and needs of the learner.		x	x	x	x	x	x			
9-2: Coaching as a professional relationship that offers tools for leading change, working with others, and managing conflict.		x	x	x	x	x	x			
9-3: Mentoring in order to promote the mentees' professional growth, enhance their skill sets, and increase their knowledge of relevant resources.		x	x	x	x	x	x			
10: Interdisciplinary/Interprofessional team building *– A supportive environment in which the skills and expertise of team members from different disciplines are acknowledged and seen as essential and synergistic. The "team" is characterized by mutual respect among stakeholders, shared leadership, equal or complementary*	**SI**	**AC**	**FE**	**LS**	**R**	**JC**	**CE**	**CB**	**SR**	**IA**

investment in the process, and acceptance of responsibility for outcomes. The MCH leaders will demonstrate a working knowledge of:										
10-1: MCH stakeholders, their roles, and contributions to a successful team.	x	x		x	x	x	x	x		
10-2: Team building concepts, including stages of team development; practices that enhance teamwork; and management of team dynamics.	x			x	x	x	x	x		
11: Working with communities and systems *– Improving the health and well-being of children, youth, families and communities in light of the complexities of many intersecting factors influencing the MCH population. Systems thinking addresses how these components interact at multiple levels, including individual organizations; the collective stakeholders; and the communities. MCH leaders will demonstrate a working knowledge of:*	**SI**	**AC**	**FE**	**LS**	**R**	**J C**	**CE**	**CB**	**SR**	**IA**
11-1: How organizations or practice setting function as systems, including business and administrative principles related to planning, funding, budgeting, staffing, and evaluating health care systems and organizations.		x		x	x	x				x
11-2: How organizations or practice setting function in relations to broader systems, including principles of systems thinking; features and issues of systems (including health economics and health policy); principles of building constituencies and engaging in collaborative endeavors; and concepts of implementation science and factors influencing use of research finding in practice.		x		x	x	x				x
12: Policy *– MCH leaders must possess policy skills, particularly in changing and competitive economic and political environments. MCH leaders will demonstrate a working knowledge of:*	**SI**	**AC**	**FE**	**LS**	**R**	**J C**	**CE**	**CB**	**SR**	**IA**
12-1: Public policy-making processes at local, state, and national levels.		x		x	x	x				x
12-2: Current public policies and private-sector initiatives that are especially relevant to MCH populations.		x		x	x	x				x
12-3: Methods for informing and educating policy makers about the needs of MCH populations and the impact of current policies on those populations.		x		x	x	x				x
12-4: Strategies for public communication on key MCH priorities.		x		x	x	x				x x

ATTACHMENT 6: Funding Tier Calculations

LEND Funding Year	Number of Long-Term Trainees reported in EHBs
July 1, 2021 – June 30, 2022	20
July 1, 2022 – June 30, 2023	16
July 1, 2023 – June 30, 2024	19
July 1, 2024 – June 30, 2025	21

Average number of Long-Term Trainees for the 4 selected years = 19

Leadership Education in Neurodevelopmental Disabilities (LEND)
Budget Justification September 30, 2026 – June 30, 2031

"The Institutional Base Salary for Clinical Faculty at the West Virginia University (WVU), Robert C. Byrd Health Sciences Center is comprised of a clinical component (i.e. Practice Plan) and a State Component. The Clinical component is paid directly to the faculty member via the Practice Plan – which is a separate entity from WVU. Therefore, for the purposes of this application, only the state component and associated percentage of effort are reflected in the personnel category. The clinical component is reflected as contractual services in "Other Expenses." The Practice Plan also employs clinical staff, and those salaries are also reflected as contractual services as well. The clinical Faculty/staff are noted below as an asterisk (.)"*

A. Senior/Key Person (YR1 $34,418; YR2 $45,929; YR3 $45,968; YR4 $46,007; YR5 $46,046). The following personnel are required for each 12-month budget period to conduct activities; first budget period is 75% salary support to account for the shortened first year grant period.

1. 65% effort in all years of the program for a Principal Investigator/Program Director, **Cassandra Miller**, EdD for the supervision of program staff, including program core faculty; linkages between LEND staff, WVUCED personnel, schools and departments participation. **Other Personnel (YR1 $180,614; YR2 $214,504; YR3 $214,811; YR4 $215,273; YR5 $215,517):**
2. 10% effort in year 1 of the program,5% effort in 2-5 of the program for Dr. **Margaret Jaynes** to assist the PI with program activities, provide mentorship for all trainees, and support the program through clinics and teaching.
3. 50% effort in year 1 and 45% effort in years 2-5 of the program for Occupational Therapist (OT), **Amy Burt**, PhD, core faculty to participate in clinical and teaching activity related to the training of LEND trainees. Responsible for directing OT activities of the program, supervise trainee participation in outreach client services, maintain trainee records.
4. 15% effort in Year 1 and 5% effort in years 2-5 of the program for Curriculum Director, **Jennifer Forester**, to coordinate and teach the Core and Policy courses.
5. 80% effort in year 1 and 100% effort in years 2-5 of the program for a Program Manager **to be hired** and assist with scheduling, course work, recruitment, hiring, data collection, and management of trainees as well as day to day operations of the program activities.
6. 100% effort in all years of the program for Administrative Secretary, **Mackenzie Karl**, to carry out all appropriate program clerical duties such as typing memorandums, reports, travel, filing and assist with tasks as assigned by the Program Director, staff and core faculty.
7. 10% effort in year 1 and 5% effort in years 2-5 of the program for Dissemination Officer, **Melina Danko**, to coordinate communication and dissemination of information related to the program and services available. In addition, she will mentor trainees about parent training, facilitate family interactions, and act as a liaison with the CED Family Grants.
8. 34% effort in all years of the program for a **self-advocate mentor** to work with trainees and families as part of the program, provide insight and assist with trainee education.
9. 20% effort in all years of the program for Accountant, **Roxanne Blosser**, to carry out the bookkeeping on program account, payroll, purchasing, and invoices of the LEND Program.
10. 5% effort in all years of the program for Web Developer, **Jeremy Bock**, to design, develop, and maintain the LEND webpages.

11. 10% effort in all years of the program for a Data Analyst, **Shannon Zimmerman**, who will maintain information systems, client data, evaluation and monitoring processes, and impact data under the direction of the Program Director.
12. 67% of 0.20FTE in all years of the program for Dietician **Kari Price**, (PT NBE) core faculty to direct the Nutrition and Feeding clinics and didactic trainings for all trainees ensuring that trainee competencies are met.

The following faculty will provide 5% effort in all years of the project as mentors for LEND trainees, assist with teaching and learning opportunities and to oversee activities of trainees:

- Dr. **David Huss***, Responsible for directing all neurodevelopmental activities of the program, supervise trainee participation in outreach client services and clinic activity, maintain trainee records.
- TBD, **Physical Therapist**, Responsible for directing all activities related to physical therapy for trainees, supervise trainee participation in outreach client services and clinic activity, maintain trainee records.

B. Fringe Costs (YR1 $49,080; YR2 $63,835; YR3 $63,926; YR4 $64,034; YR5 $64,102):
West Virginia University fringe benefit rates are set by the federal cognizant agency (US Department of Health and Human Services) at 26.5% for full-time state employees and 7.8% for Non benefits eligible (Price, Jaynes, Forester)

Fringe Costs	STATE BE Rate	PT NBE					
Social Security	7.15%	7.20%	$ 15,396	$ 18,635	$ 18,660	$ 18,695	$ 18,716
Worker's Compensation	0.22%	0.25%	$ 486	$ 581	$ 582	$ 583	$ 584
Unemployment	0.14%	0.35%	$ 390	$ 423	$ 423	$ 424	$ 425
Insurance	12.70%	0.00%	$ 21,941	$ 29,557	$ 29,601	$ 29,648	$ 29,678
Retirement	5.92%	0.00%	$ 10,228	$ 13,778	$ 13,798	$ 13,820	$ 13,834
Terminated Annual Leave	0.37%	0.00%	$ 639	$ 861	$ 862	$ 864	$ 865
	26.50%	7.80%	**$ 49,080**	**$ 63,835**	**$ 63,926**	**$ 64,034**	**$ 64,102**

C. Equipment ($0)

D. Travel (YR1 $16,380; YR2 $15,380; YRS 3-5 $6,000):

The LEND Program requires various national and regional meetings, program events and health care initiatives, held at various locations within the region, as well as remote sites to ensure input, participation, training, and attendance of faculty, staff, and trainees. Travel to AUCD for year 2 will only be partially funded and the balance will be requested from carryover funds if available.

Travel includes required **national meetings** with Maternal and Child Health Bureau, AUCD, Autism CARES Conference, car rental, mileage, hotel and meals and registration fees. 10 Day trips for program experiences/clinics/initiatives (YRS1-5 $600); Rental car ($35) plus Fuel ($25) x 10 trips = $600; Regional Conferences for program staff accompanying students (YRS 1-5 $900); Car Rental $35 plus Fuel ($30) x 3 trips $195; Registration $200 x 3 = $600; Parking and tolls $35/trip x 3 trips = 105 = Total $900/trip.

AUCD Conference PI/Co-I Travel (YR1 $3,646 (2 travelers); YR2 $1,000 (partial coverage) YRS 3-5 $1,823 1 traveler; Registration $750; Car Rental - $35/day x 3 days = $105

Fuel - $100; Per Diem 1 full day and 2 travel days @75% ($79 x .75 x 2) = $197.50; Lodging 2 days @$275/day = $550; Parking $40/day x 3 days = $120 = Total per traveler: $1,823 (rounded).

Funds allocated support program staff travel for the annual meeting. Additional funds are allocated to support trainee attendance for AUCD and will be available upon request by trainees for attendance (YRS 3-5 $1,000) are available out of the overall travel budget upon application by trainee to Program Director for participation in relevant conferences. Whenever possible, trainees shall travel with program staff. The LEND initiative is an "interdisciplinary" program requiring various meetings, program events, planning sessions and training initiatives to be held at various locations within the state to insure input, participation, training, and attendance of faculty, staff, trainees, advocates, and the health care professionals. Selected sites throughout the state with telecommunications capability will maximize information dissemination and training efforts in the most cost-effective manner.

E. Participant/Trainee Support Costs (YR1 $198,000; YRS 2-5 $176,000):
Trainees (YR1 $180,000; YRS 2-5 $160,000): At least 18 trainees will be supported by the LEND Program each year. Disciplines include but are not limited to Speech, Social Work, Rehabilitation Counseling, Audiology, Sports Medicine, Occupational Therapy, Nursing, and Physical Therapy.
YR1 18; YRS 2-5 16 Graduate Student Level Trainees: (various disciplines) will receive a stipend of $10,000 for the full academic year (39 weeks) for all years of the project. 2 Additional non-student trainees are included under consultant/contractual costs. Trainee Insurance Costs (YR1 $18,000; YRS 2-5 $16,000).

F. Other Direct Costs (YR1 $129,594; YR2 $94,085; YR3 $91,824; YR4 $91,355; YR5 $91,051): Supplies (YR1 11,778; YR2 $1,769; YR3 $758; YR4 $539; YR5 $585):
Office supplies, office related equipment (upgrade trainee equipment such as laptops, tablets, monitors, printers, video equipment, etc.) needed for program staff/trainees with item costs not exceeding $5000. Reference and Training Materials required to maintain the LEND Program and expenses relevant to the training program including duplication of research journals, curriculum development, and videotaping of clinical activity.

Consultants (YRS 1 $66,316; YRS 2-5 $43,316):
*Clinical Contracts: (YRS 1-5 $12,316) A UHA Agreement will be done for the clinical contract salary portion of David Huss for 5% effort in all years of the grant (inclusive of fringe at 26.5%).
CODA hosting support (YR1 $4,000; YRS 2-5 $1000): Funds in year 1 will be used to update data collection templates for reporting$3,000, Years 1-5 funds are allocated for support of center data collection system, shared by all center programs, (YRS1-5 $1,000).
Conference Support (YR1 $10,000): Funds are allocated to support educational conferences and experiences related to the program, to host speakers, and otherwise present training opportunities for program participants.

Family/Community Mentor (YRS 1-5 $10,000): A community family member will be recruited to serve as a mentor to trainees and will receive a stipend for participation in teaching and training activities, mentoring, and presenting to trainees ($10,000/year).
Community Social Work Mentor (YRS 1-5 $10,000): A community social worker will be recruited to serve as a mentor to trainees and will receive a stipend for participation in teaching and training activities, mentoring, and presenting to trainees ($10,000/year).
Non-Student Trainees: These trainees will complete the same program but will be recruited from the community and families.1 family trainee with a stipend of $10,000 each year of the program (YRS1-5 $10,000). 1 self-advocate trainee with a stipend of $10,000 each year of the program (YRS 1-5 $10,000)

Printing/Binding/Postage (YR1 $2,500; YR2 $1,000; YRS 3-4 $250; YR5 $150)
$300 allocated in all years of the project for mailing information to parents, trainees, and community partners Trainings/program information materials. Printing and binding costs allocated at $2,000/yr for years 1-3, $1,000 in year 4, and $500 in year 5. $500 is allocated in all years of the project for copier support costs for curriculum duplication and production of visual display teaching/learning materials.

Rent/Utilities/Telecommunications (YR1 $47,000; YR2 $46,500; YRS 3-5 $46,000)
Communication: (YR1 $2,000; YR2 $1,500; YRS 3-5 $1,000) funds are allocated for service lines, long distance charges, maintenance charges, internet connectivity, faxing charges.
Space Rental: $45,000 is allocated in all years of the project for space for the LEND Program for faculty office space and utilities for individual offices, faculty, secretary and accountant space, data collection space, trainee office space (in one large unit), and clinic and classroom/meeting space. Indirect costs are not assessed on the Rental Contribution.

Hospitality (YR1 $2,000; YRS 2-3 $1,500; YR4 $1,250; YR5 $1,000) funds are allocated for light refreshments at training, orientation, and team meetings.

G. Direct Costs (YR1- $612,332), (YR2-$610,932), (YRS3-5 $596.852)

H. Indirect Costs (YR 1- $28,308), (YR2-$30,068) (YR3-5 $30,068): West Virginia University's negotiated indirect cost rate for other sponsored programs, with its federal cognizant agency (Department of Health and Human Services), is currently set at 26%, however sponsor limits indirect costs for this application to 8%. *The U.S. Department of Health and Human Services is West Virginia University and the WVU Research Corporation's "cognizant agency" for negotiating facilities and administration (F&A) and Fringe Benefit rates to be charged on all sponsored agreements. POC Darryl W. Mayes (214) 767-3261*

Total Direct and Indirect Costs (YRS1-5 $641,000)